



THE 5TH (2024) GENERATION SURVEILLANCE OF FEMALE SEX
WORKERS, PATRONS AND WORKERS OF SITES WHERE
PERSONS MEET SEX PARTNERS OR PARTICIPATE IN SEXUAL
ACTIVITY IN JAMAICA

**Submitted to
THE MINISTRY OF HEALTH AND WELLNESS, JAMAICA**



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A-Z INFORMATION JAMAICA LIMITED
The e-Biz Centre | 2 Trafalgar Road | Kingston 5 | Jamaica | West Indies
T: (876) 927 6137 | (876) 997 9489 | F: (876) 946-0236
Email: noelwatson@azinfojam.com | info@azinfojam.com | Website:
www.azinfojam.com

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ACRONYMS

AIDS	Acquired Immune Deficiency Syndrome
ART	Anti-retroviral Therapy
BBS	Bio-Behavioural Surveillance
CSW	Commercial Sex Worker
FSW	Female Sex Worker
FP	Female Patron
HIV	Human Immunodeficiency Virus
MOHW	Ministry of Health and Wellness
MSM	Men who have Sex with Men
NERHA	North-East Regional Health Authority
NHP	National HIV Programme
PLACE	Priorities for Local AIDS Control Efforts
PLHIV	People Living with HIV
PrEP	Pre-exposure Prophylaxis
PeP	Post-exposure Prophylaxis
PW	PLACE Worker
S&D	Stigma and Discrimination
SRH	Sexual and Reproductive Health
SERHA	South-East Regional Health Authority
SRHA	Southern Regional Health Authority
UNAIDS	United Nations Programme on HIV
WRHA	Western Regional Health Authority

1. EXECUTIVE SUMMARY

Jamaica has both a generalized and a concentrated HIV epidemic. In 2021 HIV prevalence among adults was 1.3% with an estimated 30,000 persons living with HIV (PLHIV). Data on high-risk populations reveal that 13% of new HIV infections were among women of high risk and 2% among female sex workers (Figueroa et al 2020, Ministry of Health and Wellness 2019). Transactional sex was reported by 45.4% of men and 16.5% of women in 2017 compared with 53.8% of men and 23.6% of women in 2012 (Skyers et al 2017. Hope Caribbean Co. Ltd 2017).

There has been much progress made in reducing HIV prevalence among female sex workers in Jamaica. HIV prevalence among female sex workers was estimated to be 12% in 1990, 9% in 1997 (Hope Enterprises 2005), and 2005 (Duncan et al 2010), 4.9% in 2008 (Duncan ,2014), 5.4% in 2011 (Weir et al 2018) 3.2% in 2014 and 2% in 2017 (Skyers et.al. 2018). While reported condom use is very high among female sex workers with their clients, it is known that a condom is not always used with a regular client and frequently not used with the main partners of sex workers (Bailey & Figueroa 2017). The high prevalence of other STI among sex workers is consistent with the failure to use condoms consistently.

The 5th Generation Cross-sectional bio-behavioural surveillance (BBSS) among female sex workers (FSW), other female workers (FW) and female patrons (FP) at sites where people meet new sexual partners also known as PLACE 2023-4 was approved by the MOHW Ethics Committee. The research protocol reflected minor changes to the 2014 and 2017 PLACE surveys to establish HIV and Syphilis prevalence among FSW, PW and FP at risk and to gather information to continue to guide and improve HIV/STIs interventions among these women. The study was carried out between September 2023 and June 2024 in the four health regions of the Ministry of Health and Wellness, South- East, North- East, Southern and Western.

Overview of Study Design & Recruitment

Data collection consisted of three phases in keeping with the PLACE methodology. Phases 1 and 2 were conducted by the HIV Prevention Programme staff after four regional workshops where they were trained in the methodology.

Methodology: Bio-Behavioural Survey

The sampling design was a 2-stage probability sample of patrons and workers at venues identified as places where sex workers solicit clients.

- A sample of females aged 16 and older were enrolled in the study.
- Participants included female sex workers (FSW), PLACE workers (PW) and female patrons (FP).
- 1660 women meeting eligibility criteria consented to the survey, 1020 of whom reported engaging in sex work in the past 6 months (see box on the right for eligibility criteria).
- Bio-samples were collected for HIV and Syphilis testing of all eligible participants and dried blood spot for viral load assessment of the HIV positive participants.

ELIGIBILITY CRITERIA

VENUE PATRONS AND WORKERS

- Self-Identify as a Woman
- At venue selected for the survey or recruited by a peer using a recruitment coupon
- Age 18 or older or Age 16-17 if not with parent or on family errand
- Willing to be interviewed and tested for HIV and syphilis
- If HIV+, willing to provide dried blood spot for viral load assessment

Results

- Most respondents identified as female with 0.5% (n=7) identifying as Transgender.
- Participants' ages ranged from 16 - 57 years. The median age was 31 and the mean 32.
- Among sex workers failure to use condom with a main partner was way higher than failure to use condoms with a paying client.
- Interaction with HIV Programme staff was highest among current sex workers with more than a third of them reporting HIV prevention education in the last 6 months.
- Ninety five percent of current sex workers and 95.2% of workers had previously been tested while 86% of patrons had received a HIV test in the past.
- More than 90% of workers, patrons and sex workers reported easy access to condoms.
- Participation in a condom demonstration skill session was reported by 62% of workers and 36.8% of patrons and 66.6% of current sex workers.
- Knowledge and use of PrEP and PeP was low among all groups and less than 10% of sex workers, workers and patrons had used either prevention method.
- Most HIV positive participants were sex workers with a 3.2% prevalence {1.6, 4.9}.
- More than half of HIV positive participants used substances and did not use condoms with a main partner, while almost a quarter had a history of violence.
- Two thirds (67.9%) of HIV positive participants previously knew their status with 65.3% on ART and of those on ART 46.3% are suppressed.
- Rapid SD bio line syphilis test results without confirmation resulted in a 3.5% prevalence for syphilis among all participants with 4.2% among sex workers.
- The North- East region accounted for the highest prevalence of syphilis (3.0%).

2. INTRODUCTION

The HIV Epidemic in Jamaica

Jamaica has both a generalized and a concentrated HIV epidemic. In 2021 HIV prevalence among adults was 1.3% with an estimated 30,000 persons living with HIV (PLHIV) (UNAIDS Country Fact Sheet - Jamaica 2021). The Ministry of Health and Wellness HIV/STI/TB Report 2019 states that among persons reported as newly diagnosed HIV cases in 2018, 63% lived in Kingston and St Andrew, St James or St Catherine. Forty three percent (43%) of newly diagnosed HIV infections in 2018 comprised heterosexual men with a high-risk profile – multiple sex partners, having a sexually transmitted infection (STI) or being a client of a sex worker. Data on high-risk populations reveal that 13% of new HIV infections were among women of high risk and 2% among female sex workers. Additionally, 10% of all new HIV infections reported in 2018 were among men who have sex with men (Figueroa et al 2020, Ministry of Health and Wellness 2019).

Linkage to Care

The Ministry of Health and Wellness HIV estimates for Jamaica states that as many as 5,293 Jamaicans may be infected with HIV and they are unaware of their HIV status (UNAIDS, 2019). Approximately 27,324 persons have been diagnosed with HIV and most of them were linked to medical care. However, 14,069 of these persons have been lost to care and may not be on treatment. As of December 2021, 14,000 (47%) PLHIV were on anti-retroviral treatment. An estimated 80% of these persons were virally suppressed, which is an indicator of successful treatment. These estimates are based on UNAIDS models using data from the Ministry of Health including the HIV surveillance system and a variety of surveys conducted in Jamaica.

Drivers of the HIV Epidemic

The Jamaica National Strategic Plan 2020-2025 lists the factors driving the HIV epidemic in Jamaica including but not limited to the following:

- The high-risk behaviour of heterosexual men including unprotected sex with commercial sex workers, multiple sex partners and transactional sex.
- The very high HIV prevalence among men who have sex with men (MSM) due to unprotected anal intercourse with multiple sex partners.
- The high rate of bisexuality among MSM that acts as a bridge for HIV transmission into the general population.

- The high rates of other sexually transmitted infections.
- The failure to affirm the rights of sex workers, MSM and other marginalized groups and decisively combat the stigma and discrimination associated with HIV and homosexuality.

HIV Epidemic among Female Sex Workers

There has been much progress made in reducing HIV prevalence among female sex workers in Jamaica. HIV prevalence among female sex workers was estimated to be 12% in 1990, 9% in 1997 (Hope Enterprises 2005), and 2005 (Duncan et al 2010), 4.9% in 2008 (Duncan ,2014), 5.4% in 2011 (Weir et al 2018) and 3.2% in 2014. The Priorities of Local AIDS Control Efforts (PLACE) female sex workers, workers and patrons survey conducted in 2017 yielded a HIV prevalence of 2%. While reported condom use is very high among female sex workers with their clients, it is known that a condom is not always used with a regular client and frequently not used with the main partners of sex workers (Bailey & Figueroa 2017). The high prevalence of other STI among sex workers is consistent with the failure to use condoms consistently.

Underlying Determinants: Social and Cultural Factors

There are many social and cultural factors that contribute to the spread of HIV (Figueroa 2009). Sex begins at an early age. The median age of first sex for boys has decreased from 16.5 years in 1996 to 15 years in 2017, and for girls from 18.2 years in 1996 to 17 years in 2017. As many as 42% of boys and 12% of girls aged 15 – 24 years reported that they initiated sex before they were 15 years of age. If one is poor and unskilled the options for getting work are limited. Many females will therefore turn to transactional sex to survive or do better socially. Transactional sex was reported by 45.4% of men and 16.5% of women in 2017 compared with 53.8% of men and 23.6% of women in 2012 (Skyers et al 2017. Hope Caribbean Co. Ltd 2017). Transactional sex may be more prevalent than these figures suggest especially among women of lower socio-economic status. If things become desperate some women will turn to commercial sex, which has grown considerably in the past few decades and takes a variety of forms (Bailey & Figueroa, 2017). These realities will likely exacerbate the negative impact of general socio-economic challenges on the population of sex workers and further increase risky sexual practices.

Rationale for the Study

There is an urgent need in Jamaica to improve current prevention Programmes to limit the spread of HIV. Many approaches are needed to contain the epidemic including school-

based Programmes, mass media campaigns and targeted approaches for those most at risk. Prevention efforts require current information about the HIV and STIs epidemics in high-risk groups.

The current 2023-4 bio-behavioural surveillance data is critical in informing policies and Programmes to better respond to the sexual health risks of sex workers, MSM and Transgender women. This study employed the Measure evaluation's (priority locations for AIDS control efforts (PLACE) methodology that has been used in Jamaica since 2002 and is described in the literature (Virkud, et.al., 2017; Weir, et.al., 2018; Zalla, et.al., 2019)

The study explores the following questions:

- What are the characteristics of venues where people meet new sexual partners in Jamaica?
- Where are venues located?
- What evidence is there of HIV prevention Programme efforts at the venues?
- Are condoms available onsite?
- Are HIV prevention messages displayed onsite?
- What is the prevalence of HIV and Syphilis among women at venues identified as places where people meet new sexual partners?
- What is the prevalence of HIV and Syphilis among women who exchange sex for money?
- What are the socio-demographic and behavioural characteristics of women who exchange sex for money?
- What is the extent to which women who exchange sex for money have access to HIV prevention Programmes?
- What are the characteristics of those infected with HIV?
- What are the characteristics of those infected with other STIs?

Objectives

The specific objectives of the study were:

- I. To determine prevalence of HIV and *Syphilis* among high risk women.
- II. To measure proximate & underlying determinants of HIV and STI transmission
- III. To monitor Programme coverage
- IV. To establish closer contacts with key populations to expand prevention and treatment services
- V. To determine effectiveness of prevention interventions among FSW
- VI. To estimate female sex workers population size

VII. To understand social and sexual networks of FSW.

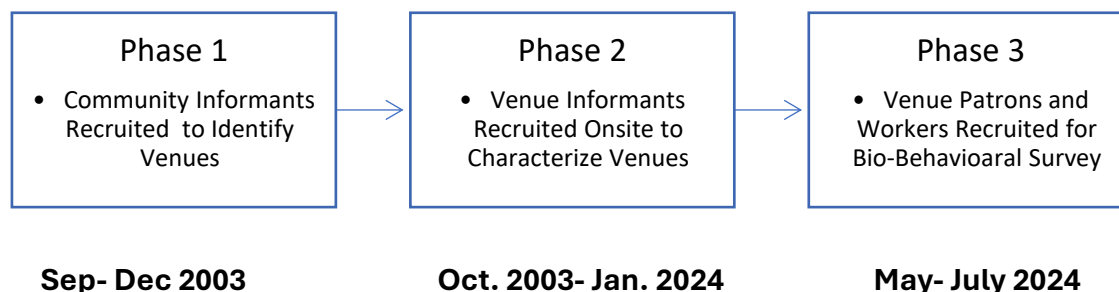
METHODOLOGY

Overview of Study Design

This is a cross-sectional quantitative survey where data collection consisted of three phases in keeping with the PLACE methodology. The figure below describes the three phases of the study. Subsequent sections of the report describe the methods and findings for each phase. The study was carried out between September 2023 and June 2024 in the four health regions of the Ministry of Health and Wellness, South- East, North- East, Southern and Western.

Each phase was implemented at the parish level at different times. All parishes had completed phase one by December 2023 and phase 2 by January 2024. Logistical issues led to a delay in the implementation of phase 3 until May 2024 beginning in the North- east region. Data collection for phase 3 ended in July 2024.

Figure 1 Phases of PLACE Study



COMMUNITY ENGAGEMENT AND ETHICAL CONSIDERATIONS

Key Messages

- Community engagement was critical to success of study.
- Implementation followed ethical standards and data confidentiality was maintained.
- Study was approved by the MOHW Ethics Committee
- 50+ Peer educators were engaged in the study.

The 5th Generation Cross-sectional bio-behavioural surveillance (BBSS) among female sex workers (FSW), other female workers (FW) and female patrons (FP) at sites where people meet new sexual partners also known as PLACE 2023-4 was approved by the

MOHW Ethics Committee. The research protocol reflected minor changes to the 2014 and 2017 PLACE surveys to establish HIV and Syphilis prevalence among FSW, PW and FP at risk. It was also constructed to gather information to continue to guide and improve HIV/STIs interventions among these women (see Appendix 1 for full research protocol).

Overview of Recruitment

Over 3000 people participated in the PLACE study either as community informants (1270) or venue informants (342) or participants in the bio-behavioural survey (1660). Figure 3 summarizes all recruitment into the study across the three phases.

Figure 2 Summary of Recruitment

1270	• Community Informants Asked "Where Do People Meet New Sex Partners?"
617	• Venues Identified In All Parishes
600	• Venues Vists Attempted
342	• Unique Venues Found Operational
1660	• Probability Sample of Patrons and Workers Interviewed and Tested at 281 Venues including 82 invited by coupon
1020	• Current FSW Interviewed and Tested from Among 1660 Patrons and Workers

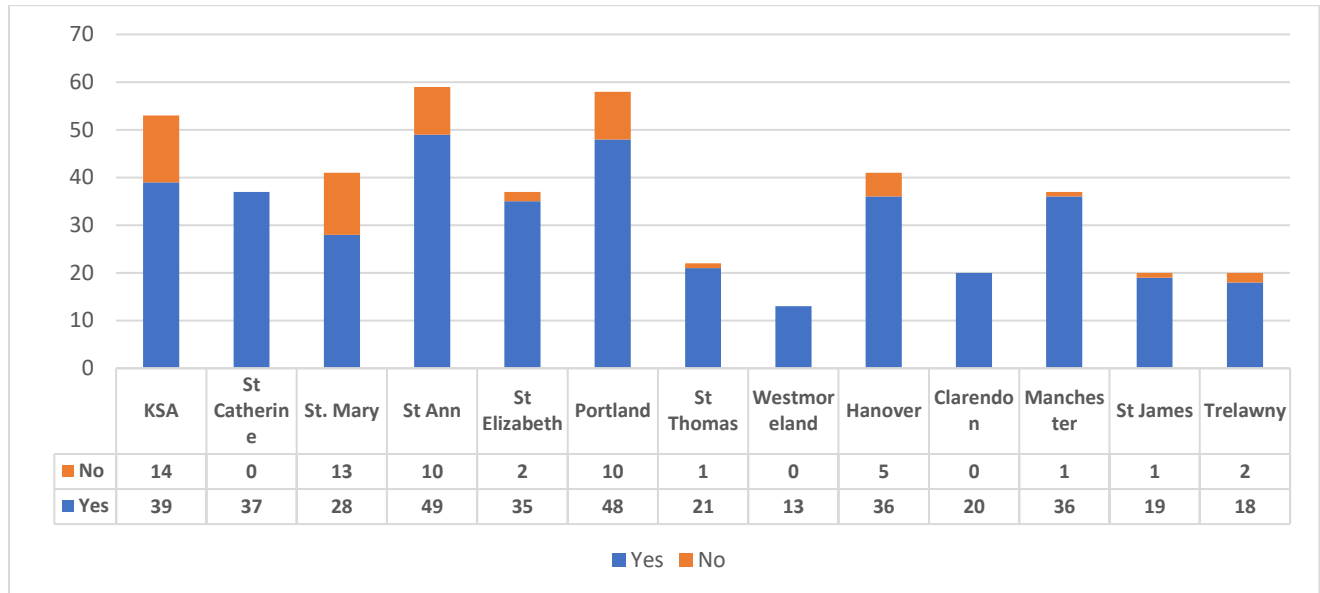
See appendix 2 for details of the site identification and verification stages

Operational Venues

A wide variety of sites emerged from the Community Informant and Site verification phases as places where people meet new sex partners. A total of 617 sites were identified in the 14 parishes in Jamaica; of these 458 were operational and 399 were confirmed as places where female sex workers solicit. Figure 3 below shows the number of operational venues identified in each parish and whether sex work was confirmed or not.

St Ann (49), Portland (48) and KSA (39) recorded the highest number of operational sites during the site verification stage. Westmoreland recorded the lowest number of verified sites (13).

Figure 3 Number of Sites with Sex Work by Parish



As there was an anticipated protracted delay between the time of Site verification (October 23- January 2024) and the implementation of the survey, A National Count of Females at verified sites on a busy night was conducted on the last weekend of February 2024 to provide targets for each parish. The numbers of females counted on a busy night decreased or remained the same in all parishes except KSA where it increased over the numbers recorded in the site verification. The results of the national count were used as recruitment targets for each parish.

BIO-BEHAVIOURAL SURVEY: ELIGIBILITY, BIO-DATA COLLECTION, SAMPLING, DATA ANALYSIS AND DEFINITION OF SEX WORK

The sampling design for phase three (the survey stage) was a two-stage probability sample of patrons and workers at venues identified as places where sex workers solicit clients.

Eligibility

- 1660 women meeting eligibility criteria consented to the survey. 1020 reported engaging in sex work in the past 6 months.
- Bio-samples were collected for HIV and Syphilis testing of all eligible participants and dried blood spot for viral load assessment of the HIV positive participants.
- In Phase 3, a sample of females aged 16 and older were enrolled in the study. Participants included female sex workers (FSW) other PLACE workers (PW) and female patrons (FP). See eligibility criteria in the box on the right.
- Bio-data collection consisted of rapid HIV and Syphilis tests. HIV positive results were confirmed with dried blood spot testing.

ELIGIBILITY CRITERIA

VENUE PATRONS AND WORKERS

- Self-Identify as a Woman
- At venue selected for the survey or recruited by a peer using a recruitment coupon
- Age 18 or older or Age 16-17 if not with parent or on family errand
- Willing to be interviewed and tested for HIV and syphilis
- If HIV+, willing to provide dried blood spot for viral load assessment

Appendix 2 contains details of the testing procedures.

Sampling Design

Selection of Venues and participants

During the time lag between site verification and the PLACE surveys some sites disappeared and other new sites appeared. The initial sample strategy was to visit all the sites with 20 or more sex workers, 50% of sites with 5-19 sex workers, and 10% of sites with less than 5 sex workers. By the time the survey began many verified sites were no longer operational and others had emerged. The survey teams therefore sought to visit all sites with 5 or more workers and a sample of sites with less than 5 workers. A corrected sampling frame that updated the list of sites was not formally developed, however, an effort was made to find all venues with 5 or more FSW including new venues and visit as many sites with less than 5 FSW as possible. A total of 181 sites were visited for the survey. Of these 104 were sites that were on the original list (including 58 sites with less than 5 FSW and 46 sites with 5 or more FSW).

A two-stage cluster sample of venue patrons and workers was identified for the bio-behavioural survey. Additionally, a convenience sample of non-venue-based female sex

workers was recruited through peer referral. The first stage was a probability sample of operational venues, with oversampling of venues with a higher number of sex workers. The second stage was selection of individuals within the selected venues.

All women at the sites were invited to participate in the survey. Coupons were given to gatekeepers at the sites as well as the female to invite females who solicit remotely to participate in the survey. Details of the sample selection are provided in appendix 2.

Data Analysis

Weighting was used in the analysis of the data from the survey to enhance the representativeness of the sample. Survey weights were calculated based on the probability that a respondent was interviewed. The probability that a respondent was interviewed was estimated by the product that the venue where she was interviewed was sampled (stage 1) and the probability that she was sampled from among the women at the venue (stage 2). A respondents' survey weight was 1 divided by the probability of selection.

Definition of Workers, Patrons and Sex Worker

Participants who reported sex work within the past 6 months (n=1020) were categorized as current sex workers. Recruited women were asked several questions about sex work. Those who reported receiving cash in exchange for sex either in answer to questions asked and recorded by the interviewer or to questions asked by the interviewer but recorded privately by the respondent on the tablet (to reduce self-presentation bias) were categorized as having engaged in sex work.

When weighted the total sample was 4414 comprised of 2838 workers and 1576 patrons. Of these, 59% met the criteria for recent sex work. Both the survey (unweighted) and weighted results are provided in table 1.

Table 1: Categories of Participants

Current Sex Worker	Workers			Patrons			Total		
	Survey	Weighted		Survey	Weighted		Survey	Weighted	
	N	%	N	N	%	N	N	%	N
Yes	896	79.4	2253	124	23.1	364	1020	59.3	2617
No	289	20.6	585	351	76.9	1212	640	40.7	1797
Total	1185	100.0	2838	475	100.0	1576	1660	100.0	4414

Population Size Estimates

- The survey was a national probability time-location sample of women at venues where people meet new sexual partners with known probability of selection of each participant. The study was implemented in all parishes, including urban and rural areas.
- The estimates from the PLACE survey are based on the probability that a person was interviewed at a venue and her corresponding sampling weight (calculated as 1 divided by the probability that she was sampled.) See sampling design section above and appendix 2.
- This initial cross sectional estimate was increased based on the responses to questions about how frequently the respondent visited the venue and extrapolating the cross-sectional estimate to the number expected over the entire month. The six months estimate also included adding in additional new women based on the number of women who reported engaging in sex work for less than 6 months at the time of the survey. Consequently, the number of sex workers that a programme could reach over the course of 6 months is greater than the number that are reachable on a given night or over the course of a month. Some people do not visit the venue every night, requiring multiple visits to the venue to reach these women. Other women are new to sex work, require multiple visits over the course of several months to reach them.

During phase 3 data collection the interviewers counted the number of dancers, street sex workers, workers and patrons at each site before each interview. At the time of recruitment into the survey each participant was characterized into one of these groups. The maximum number in each group was taken as the best estimate of the potential number of participants in that group. Although the protocol was to interview every female at the site, it was not possible to interview each person. Based on the number interviewed at a site and the number that were counted at the site a probability of selection for each person at the site was calculated.

RESULTS

Number of Participants Recruited by Region and Parish

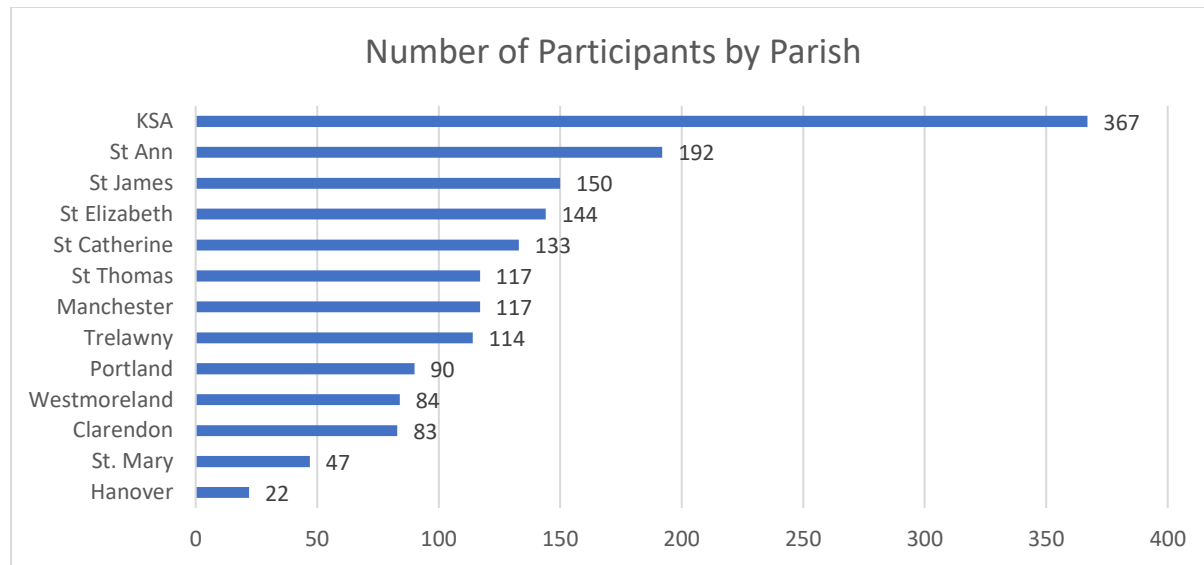
The survey was conducted in all 14 parishes in Jamaica grouped into the four health regions of Ministry of Health and Wellness. Most participants were recruited in the South-East region at 37.2% (n=617) followed by the Western region at 22.3% (370) the Southern region 20.7% (n= 344) and the North-east region 19.8% (n=329) of respondents (table 2).

Table 2 Participants by MOHW Regions by Sex Worker Status in Past 6 Months

	Venue Workers			Venue Patrons			Grand Total	
	Sex Work in Past 6 Months			Sex Work in Past 6 Months			All	%
	Yes	No	Total	Yes	No	Total		
Southeast	288	131	419	43	155	198	617	37.2
Northeast	174	45	219	27	83	110	329	19.8
Western	194	66	260	37	73	110	370	22.3
Southern	240	47	287	17	40	57	344	20.7
Total	896	289	1185	124	351	475	1660	100.0

The largest number of participants were recruited in Kingston and St. Andrew 22.1% (n=367) followed by St. Ann with 11.6% (n= 192) then St James 9% (n= 150) and St. Elizabeth with 9% (n=144). The lowest number of participants were recruited in Hanover 1.3% (n= 22) (Figure 4).

Figure 4: Number of Participants by Parish



Recruitment by Type of Site

There were 10 main types of sex work sites (identified in the site verification stage) where the survey was conducted. These included exotic clubs/ massage parlours/ sex shops, street sex work sites, outdoor hang out spots/parks, bars, night clubs, restaurants/fast food establishments/internet cafes, hotels/ motels/ guest houses, malls/ plazas, campus sites and private sex work arrangements. The category other was also included for any sites that did not fit in any of the 10 categories. The largest proportion of participants were recruited at exotic clubs (25.7% N=427) and night clubs (17.3% N=288) followed by street sex work sites (17% N=267). Hotel motels and those sites categorised as other accounted for 9% (N=15) of the sample (table 3).

Table 3 Types of Sex Work sites where participants were recruited by parish

PARISH OF SITE OR EVENT	TYPE OF SITE											TOTAL
	Bar	Night Club	Exotic Club / Sex Shop	Fast Food Restaurant. Internet Cafe	Outdoor Hang Out Spot Park	Street Sex Work Site	Hotel / Motel / Guest-house	Campus Site	Mall/ Plaza	Private	Other	
St Ann	2	22	116	0	0	49	2	0	0	0	1	192
	1.0%	11.5%	60.4%	0.0%	0.0%	25.5%	1.0%	0.0%	0.0%	0.0%	0.5%	100%
Clarendon	27	3	24	0	0	22	0	0	4	0	3	83
	32.5%	3.6%	28.9%	0.0%	0.0%	26.5%	0.0%	0.0%	4.8%	0.0%	3.6%	100%
Hanover	16	0	0	0	6	0	0	0	0	0	0	22

	72.7%	0.0%	0.0%	0.0%	27.3%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	100%
KSA	133	30	61	0	61	64	12	0	2	4	0	367
	36.2%	8.2%	16.6%	0.0%	16.6%	17.4%	3.3%	0.0%	0.5%	1.1%	0.0%	100%
Manchester	41	18	50	0	4	4	0	0	0	0	0	117
	35.0%	15.4%	42.7%	0.0%	3.4%	3.4%	0.0%	0.0%	0.0%	0.0%	0.0%	100%
Portland	21	1	21	3	30	8	0	6	0	0	0	90
	23.3%	1.1%	23.3%	3.3%	33.3%	8.9%	0.0%	6.7%	0.0%	0.0%	0.0%	100%
St Catherine	6	8	55	0	0	64	0	0	0	0	0	133
	4.5%	6.0%	41.4%	0.0%	0.0%	48.1%	0.0%	0.0%	0.0%	0.0%	0.0%	100%
St Elizabeth	25	105	12	0	1	0	1	0	0	0	0	144
	17.4%	72.9%	8.3%	0.0%	0.7%	0.0%	0.7%	0.0%	0.0%	0.0%	0.0%	100%
St James	27	33	34	0	2	29	0	0	6	8	11	150
	18.0%	22.0%	22.7%	0.0%	1.3%	19.3%	0.0%	0.0%	4.0%	5.3%	7.3%	100%
St. Mary	0	0	43	0	0	0	0	4	0	0	0	47
	0.0%	0.0%	91.5%	0.0%	0.0%	0.0%	0.0%	8.5%	0.0%	0.0%	0.0%	100%
St Thomas	75	4	4	0	34	0	0	0	0	0	0	117
	64.1%	3.4%	3.4%	0.0%	29.1%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	100%
Trelawny	21	62	4	0	8	19	0	0	0	0	0	114
	18.4%	54.4%	3.5%	0.0%	7.0%	16.7%	0.0%	0.0%	0.0%	0.0%	0.0%	100%
Westmoreland	64	2	3	6	1	8	0	0	0	0	0	84
	76.2%	2.4%	3.6%	7.1%	1.2%	9.5%	0.0%	0.0%	0.0%	0.0%	0.0%	100%
Total	458	288	427	9	147	267	15	10	12	12	15	1660
% of Total	27.6%	17.3%	25.7%	0.5%	8.9%	16.1%	0.9%	0.6%	0.7%	0.7%	0.9%	100%

DEMOGRAPHIC CHARACTERISTICS

Most respondents identified as female with 0.5% (n=7) identified as Transgender. Participants' ages ranged from 15 to 57 years. The median age was 31 and the mean 32 years. Most participants 71% (n=1,178) had obtained a secondary level education, with 10.9% (n=181) each having completed Junior high or All-age and tertiary education (table 4). A larger proportion of those who reported no paid sex in the last 6 months (6.0% n= 99) had tertiary education than those who reported paid sex (4.9% n=82). See table 4.

Table 4: Participants Educational Level

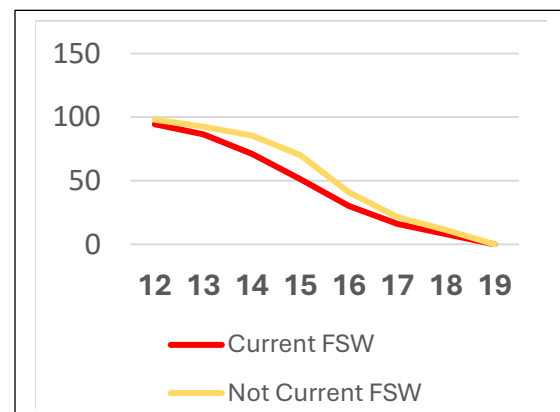
PARTICIPANTS EDUCATIONAL LEVEL	Current Sex Worker	Not Current SW	WORKER	PATRON	TOTAL
Did Not Complete Primary	12	3	15	0	15
	0.7%	0.2%	0.9%	0.0%	0.9%
Completed Primary	75	26	79	22	101
	4.5%	1.6%	4.8%	1.3%	6.1%
Junior High /All Age	110	71	130	51	181
	6.6%	4.3%	7.8%	3.1%	10.9%
High School/ Secondary School	741	437	847	331	1178
	44.6%	26.3%	51.0%	20.0%	71.0%
Tertiary	82	99	113	68	181
	4.9%	6.0%	6.8%	4.1%	10.9%

The mean and median ages of sexual initiation of participants were both 16; SD =2. Lower ages of sexual initiation were recorded among participants who reported sex for cash (Table 5 & figure 5).

Table 5: Age at First sex

Age at First Sex	Current FSW % (N)	Not Current FSW % (N)
≤12 years	94.4% (55)	98% (18)
13 years	86.6% (73)	92.5% (30)
14 years	71.2% (120)	85.5% (48)
15 years	51.3% (220)	70.1% (108)
16 years	30.1% (234)	41.0% (180)
17 years	16.0% (143)	21.3% (114)
18 years	8.1% (83)	11.2% (73)

Figure 5: Age of First



Sixty eight percent (68.4%) of respondents who reported a main partner had a main male partner while 10.4% had a main female partner. A Higher portion of those who reported current paid sex (7.9%) had female sex partners than those who did not (2.5%). Of participants who reported paid sex in the last 3-6 months 31.2% (n=270) reported to be married or living with a main partner (Table 6).

Table 6: Partner relationships

	Current FSW % N	Not current FSW % N	Total
Has a main Male Partner	39.4% (654)	29% (482)	68.4% (1136)
Has a main female Partner	7.9% (131)	2.5% (41)	10.4% (172)
Married or living with a mani partner	31.2% (270)	23.6% (204)	54.7% (474)

The demographic characteristics of participants who reported the exchange of sex for cash are recorded by MOHW regions in table 7. Current sex workers in the South- East region had a lower proportion who attained tertiary education than all other regions.

Table 7: Demographic Characteristics of Women Reporting Exchanging Sex for Money in the past 6 Months by Region (weighted percentage distribution)

Characteristic	South-East	North-East	Western	Southern	All
Number Interviewed	331	201	231	257	
Age					
16-17	1.1	0.0	0.4	0.3	0.5
18-19	5.8	4.2	3.1	3.6	4.3
20-21	7.2	3.8	10.5	4.8	6.9
22-24	9.2	7.1	17.6	8.5	10.9
25-29	21.3	20.0	19.3	20.4	20.3
30-39	38.7	37.4	35.5	36.6	37.2
40+	16.8	27.5	13.6	25.9	20.1
	100.0	100.0	100.0	100.0	100.0
Mean Age in Years	31.6	33.6	29	33.5	32.1
Median Age in Years	30	33	30.3	33	31
Sex / Gender					
Born Female & Self Identify as Female	99.7	99.4	99.4	99.4	99.4
Born Male & Self Identify as Female	0.2	0.5	0.5	0.5	0.5
Refused	0.2	0.1	0.1	0.1	0.1
	100.0	100.0	100.0	100.0	100.0
Educational Attainment					
Less than Primary	0.5	0.8	0.8	0.8	0.8
Primary	3.6	5.3	5.3	5.3	5.3

Characteristic	South-East	North-East	Western	Southern	All
Junior High / All Age	11.2	9.6	9.6	9.6	9.6
High / Secondary School	78.3	75.4	75.4	75.4	75.4
Tertiary	6.4	9.0	9.0	9.0	9.0
	100.0	100.0	100.0	100.0	100.0
Marital Status					
Never Married or Lived with a Partner	53.3	49.1	49.1	49.1	49.1
Currently Married or Living with a Partner	23.7	23.6	23.6	23.6	23.6
Not Currently Married or Living with a Partner	23.0	27.3	27.3	27.3	27.3
	100.0	100.0	100.0	100.0	100.0
Number of Pregnancies*					
0	9.2	14.2	14.2	14.2	14.2
1	26.3	22.1	22.1	22.1	22.1
2	28.5	24.9	24.9	24.9	24.9
3	13.2	15.6	15.6	15.6	15.6
4	7.7	9.7	9.7	9.7	9.7
5+	15.3	13.5	13.5	13.5	13.5
	100.0	100.0	100.0	100.0	100.0
*excludes 7 respondents who were born male and refused to answer the gender question.					

Twenty seven percent of current sex workers reported that they got into sex work because they got pregnant and did not finish school. Most current sex workers (85.8%) had at least one pregnancy, 24.9% had two 15.6% three pregnancies with an additional 23.2% having had four or more pregnancies. A higher proportion of current sex workers in the South-East region had five or more pregnancies (table 7).

Fifty percent (50% n=278) of those who reported current sex work also reported being in the business between 1-5 years while 35% (n=196) had been soliciting for over 5 years.

SEXUAL BEHAVIOUR

Condom Use with Main and Paying Partner

Ninety percent of participants who reported exchanging sex for money in the last 6 months (recent sex work) also reported sex work in the last three months while 79.4% reported two

or more sex partners in the last 4 weeks. Respondents were asked if they used a condom at the last sexual encounter with their main partner; 37% of workers and 59% of patrons and 36% of current sex workers reported they had not. This compares unfavourably with 7.4% no condom use with a new paying client among current sex workers. Of the respondents that reported anal sex in the last 12 months 11.7% of workers and 10.8% of patrons and 11.6% of current sex workers reported they had not used condoms (Table 8).

Table 8: Risky Sexual Practices

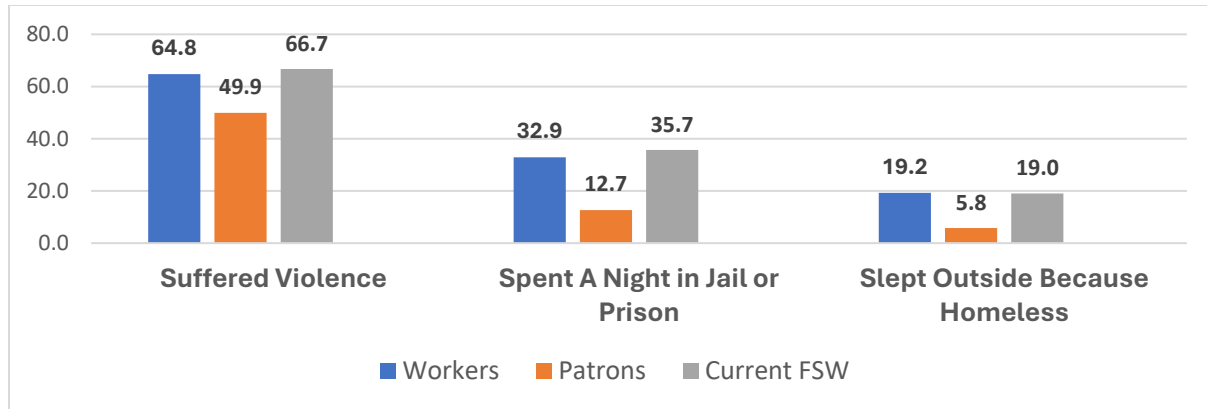
Sexual Practice	Workers	Patrons	Current FSW
No Condom Main Partner	37% (694)	59% (329)	36% (576)
Sex Work in Past 3 months	73.7% (713)	18% (96)	90.7% (809)
Unprotected Anal Sex in Past Year	11.7% (115)	10.8% (33)	11.6% (107)
No condom last sex with regular paid partner	2.5% (31)	1.2% (8)	3.4% (38)
2+ partners in the past 4 weeks	68.3% (756)	23.5% (88)	79.4% (788)

Vulnerability factors

Participants were asked about incidents of violence with their main partner or other sexual partners. Workers reported a 64.8% prevalence of experiencing violence including having been slapped, choked, raped or controlled by a sex partner while 49.9% of patrons and 66.7% of current sex workers reported violent experiences with a sex partner (figure 11).

Ever having spent a night in jail was reported by 32.9% of workers as well as 12.7% of patrons and 35.7% of current sex workers. Homelessness had been experienced by 19% of workers and current sex workers as well as 5.8% of patrons (figure 6).

Figure 6: Factors that lead to vulnerability

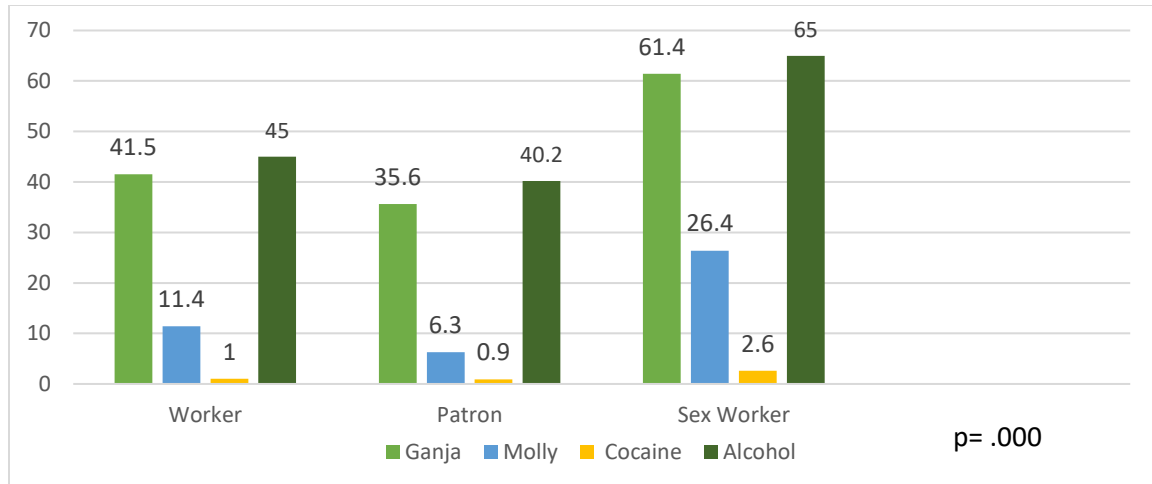


Substance use including recent use and frequency of alcohol consumption (6+ drinks in last 4 weeks) and recent use of Ganja/ marijuana, ecstasy/molly and cocaine (in the past 6 months). Sex workers reported significantly higher use of all substances except cocaine than other groups with 65% alcohol, 61.4% marijuana, 26.4% molly and 2.6% use (table 9 & figure 7).

Table 9: Substance Use among Workers, Patrons and Sex Workers

Substance Used	Workers % N	Patrons % N	Current FSW % N
Ganja	41.5% (646)	35.6% (177)	61.4% (245)
Molly	11.4% (287)	6.3% (37)	26.4% (269)
Cocaine	1% (27)	0.9% (6)	2.6% (27)
Alcohol	45% (728)	40.2% (204)	65% (661)

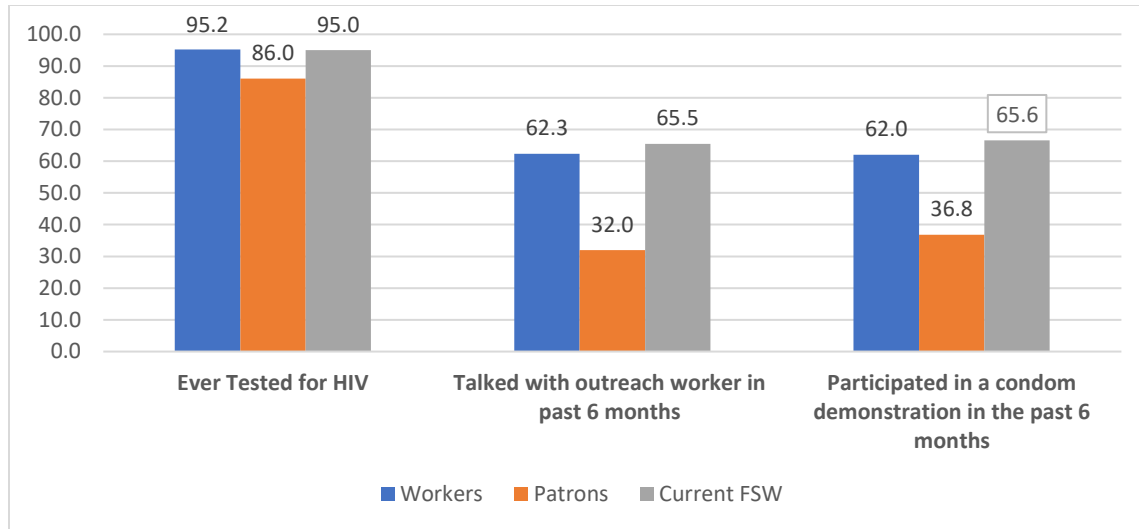
Figure 7: Substance Use among Workers, Patrons and Sex Workers



EXPOSURE TO HIV PREVENTION

Interaction with a HIV prevention outreach worker in the last 6 months was highest among current sex workers 65.5% followed by 62.3% of workers while 32% of patrons had done so. Participation in a condom demonstration skill session was reported by 62% of workers and 36.8% of patrons and 66.6% of current sex workers. Most participants had been tested in the past. Ninety five percent of current sex workers and 95.2% of workers had previously been tested while 86% of patrons had received a HIV test in the past (figure 8).

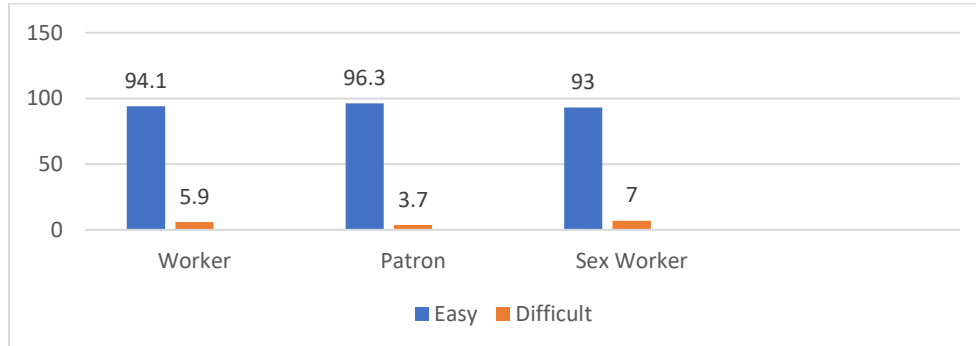
Figure 8: Exposure to HIV Prevention Programmes



Access to Condoms

Condom access for participants was reportedly easy; greater than 90% of workers, patrons and sex workers reported easy access (figure 9)

Figure 9: Access to condoms by Workers, Patrons and Sex Workers



Knowledge of Pre-exposure and Post-exposure Prophylaxis

Pre-exposure prophylaxis (PrEP) is an additional prevention tool for HIV negative people at substantial risk of HIV exposure who are not always able to have safer sex. Post-exposure prophylaxis (PEP) is the use of antiretroviral drugs after a single high-risk event to stop HIV seroconversion.

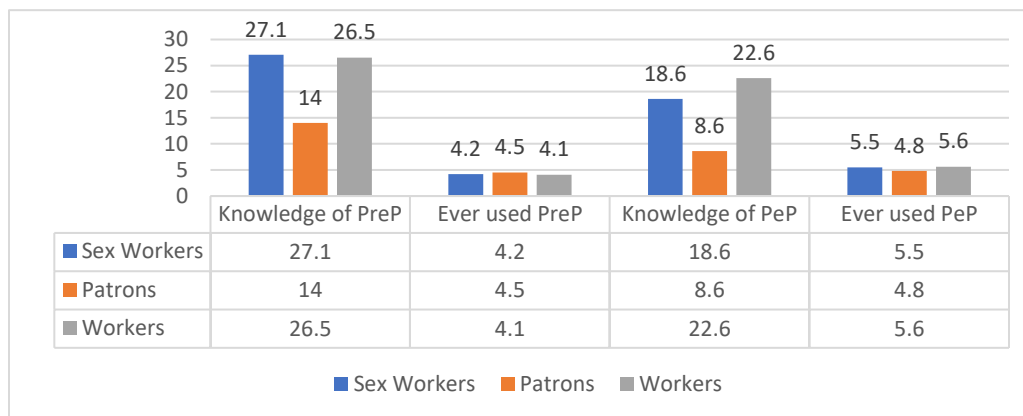
Twenty seven percent (27.1%) of sex workers knew about PrEP while 18.6% had used it; among patrons 14% knew of PrEP and 8.6% had used it while among workers 26.5% knew

of PrEP and 4.1% had used it. Regarding knowledge and use of PeP, 18.6% of sex workers knew of it while 5.5% had ever used it while 22.6% of workers knew of Pep and 5.6% had used it compared to 8.6% of patrons who knew of PeP while 4.8% had used it (table 10 & figure10).

Table10: Knowledge and ever use of PrEP and PeP

Category	Workers % N	Patrons % N	Current Sex Workers % N
Knowledge of PrEP	26.5% (314)	14% (66)	27.1% (277)
Ever use of PrEP	4.1% (13)	4.5% (3)	4.2% (13)
Knowledge of PeP	22.6% (268)	8.6% (41)	18.6% (233)
Ever Use of PeP	5.6% (15)	4.8% (2)	5.5% (12)

Figure 10 Knowledge and ever use of PrEP and PeP



HIV PREVALENCE AND TREATMENT CASCADE

In this component, both the weighted and unweighted results of the HIV and syphilis testing for participants in the survey are presented.

There were 29 HIV positive results recorded in the survey, 22 of whom were workers and seven (7) Patrons. Twenty-five (25) current sex workers tested positive, this represents a weighted prevalence of 3.2% of the participants who reported exchanging sex for cash in the last 6 months [1.6, 4.9]. The prevalence among workers is 2.2% [1.0, 3.3] and 2.4% [0.1, 4.8]

of patrons. Higher weighted prevalence was recorded among participants aged 25 years and older in all categories 3.7% among current sex workers, 3.0% among patrons and 2.7% among workers (table 11).

Table 11: HIV Prevalence by Age, Patron, Worker and Sex Worker Status

	Workers			Patrons			Sex Workers		
	N HIV+	%	95% CI	N HIV+	%	95% CI	N HIV+	%	95% CI
All	22	2.2	1.0, 3.3	7	2.4	0.0,4.8	25	3.2	1.6, 4.9
Age <25	3	0.6	0,1.3	1	1.4	0.0,4.3	3	1.8	0.0, 4.5
25+	19	2.7	1.2,4.2	6	3.0	0.0,6.1	22	3.7	1.8,5.6

Sex workers in the North-East region recorded the highest HIV prevalence 4.0% [0.3, 5.8] followed by the Western region with 3.6% [0.1, 7.0] then the South-East with 3.0 [0.3, 5.8] and the Southern with 2.1 [0.0, 5.0]. Patrons in the Western region recorded the highest prevalence of 4.5% [0.0, 11.1] while workers in the North-East 3.2% [0.4, 6.1] recorded a higher prevalence than workers in other regions (table 12) .

Table 12: HIV Prevalence by Regions, Patron, Workers and Sex Worker Status

Regions	Workers			Patrons			Current Sex Workers		
	N HIV+	%	95% CI	N HIV+	%	95% CI	N HIV+	%	95% CI

South-East	6	1.9	0.0,	4.0	3	1.7	0.0	3.9	7	3.0	0.3	5.8
North-East	6	3.2	0.4,	6.1	1	2.6	0.0	8.4	6	4.0	0.5	7.5
Western	8	2.1	0.5,	3.7	2	4.5	0.0	11.1	9	3.6	0.1	7.0
Southern	2	1.5	0.0,	4.0	1	1.4	0.0	3.6	3	2.1	0.0	5.0

Characteristics of HIV Positive Participants

Fifty five percent (55%) of HIV positive participants who reported recent sex work did not use condoms with a main partner and 24% had a history of partner violence. Additionally, 50% used substances (table 13).

Table 13: Characteristics of HIV Positive Participants

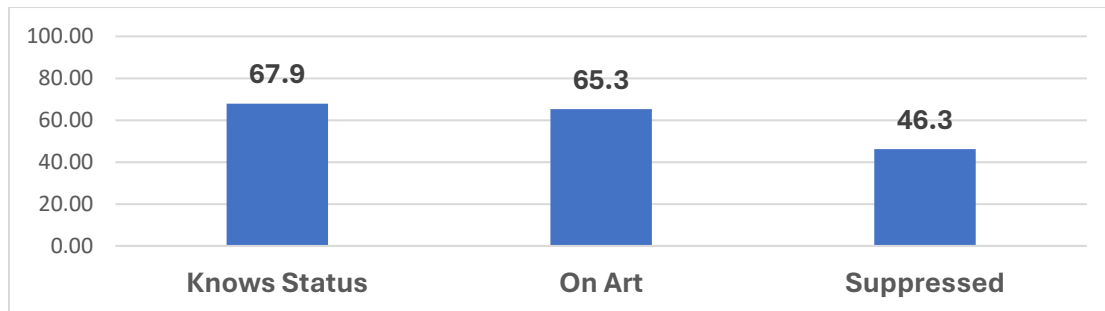
Characteristics	Positive current Sex work		Positive Other	Sub-total	Positive Patrons	
	N	%	N	N	N	% of sample
No Condoms with main partner	17	55.0%	1	18	2	7.0%
Received money for sex in the last 3 months	17	58%	2	19	3	10.0%
Anal sex no condom	5	26.3%	0	5	0	0.0%
Last paid sex no condom	1		0	1	0	0.0%
History of violence, partner control & rape	8	24.0%	0	8	1	4.0%
Substance/ Drug use	14	50.0%	3	17	5	15.0%

HIV Treatment Cascade

The 29 participants who tested positive in the survey consisted of 22 workers and seven (7) Patrons, 25 of whom reported current sex work. Dried blood spot testing was used for bio-data collection to discover viral loads. Results indicate that 18 (67.9%) persons previously

knew their status with 17 (65.3%) on anti-retroviral therapy (ART) and of those on ART 46.3% (n=8) are suppressed (figure 11)

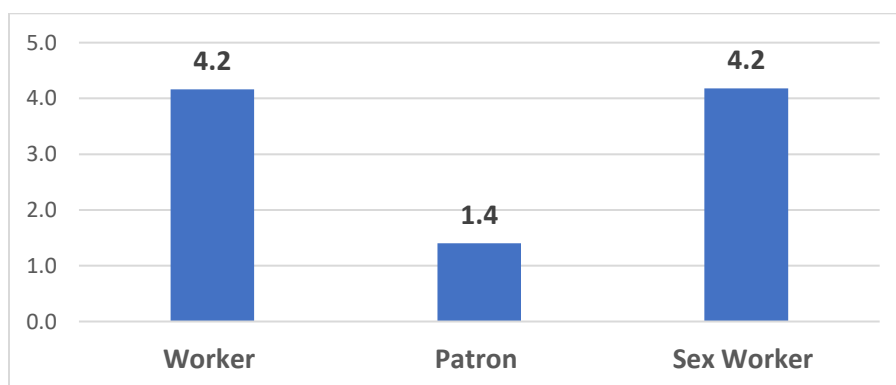
Figure 11: Treatment Cascade of HIV Positive Participants



SYPHILIS RESULTS

The rapid test for syphilis gives a reactive result for everyone who has ever been infected with syphilis and requires a confirmatory test to diagnose active untreated syphilis. There were 59 reactive syphilis test results from the sample of 1660 participants indicating recent or past syphilis infection comprising 52 workers and seven (7) patrons. This indicates a tentative prevalence of 3.5% of the entire sample with 4.2% [2.6, 5.7] for workers (n=1185) 1.4% [0.3, 2.5] among patrons(n=475) and 4.2% [2.5, 5.9] among women reporting recent sex work (n=1020). These results are illustrated in figure 12.

Figure 12: Participants with Reactive Syphilis Tests



The highest prevalence of syphilis (7.4%) was recorded among those who were 40 years and older. The region that recorded the highest prevalence of syphilis among the participants was the North-East region with a prevalence of 3.0% (table14)

Table 14: Participants with a Reactive Syphilis Test by Age, Region, Sex Worker, Patron and Worker Status

	Survey		Weighted		
Patron or Worker	N	N Reactive	%	95% CI	
• Worker	1185	52	4.2	2.6	5.7
• Patron	475	7	1.4	0.3	2.5
Current Sex Worker					
• Yes	1020	44	4.2	2.5	5.9
• No	640	15	1.7	0.8	2.6
Age Group					
• 16-17	26	0	.	.	.
• 18-19	101	3	1.5	0.0	3.6
• 20-21	135	2	0.8	0.0	2.0
• 22-24	200	2	2.4	0.0	6.1
• 25-29	315	4	0.7	0.0	1.5
• 30-39	536	23	3.8	1.7	5.9
• 40+	346	25	7.4	3.7	11.1
Region					
• Southeast	612	9	1.8	0.2,3.3	
• Northeast	320	7	3.0	0.2,5.1	
• Western	368	10	2.7	0.3, 5.1	
• Southern	343	3	1.5	0, 3.6	

POPULATION SIZE ESTIMATES

Three estimates of the size of the female sex work population based on surveys of patrons and workers during the bio-behavioural survey are provided:

The summary of estimates of the population of female sex workers based on the count of females during the site verification and survey data collection phases are presented in table 15 as follows:

- 2,617 - the number of female sex workers who can be reached on a busy night at venues where people meet new sexual partners.

- 11,433 - the number of sex workers who can be reached at the venues over the period of a month.
- 14,000 - the number who can be reached at venues over the course of 6 months.

Table 15 : Population Size Estimates Female Sex workers

Description	Estimate	Confidence Interval
Number of women in Jamaica who exchange sex for money who:		
• Can be reached on a busy day/night at venues	2,617	2165, 3069
• Can be reached over the course of a month at venues	11,433	8852, 14014
• Can be reached over the course of 6 months at venues	14,000	11,000 – 15,000

Estimates from Site Verification

The size estimates from the venue verification study are shown below in table 16. The estimates are based on the information provided at 412 venues that were operational and provided answers to the size estimation questions.

Table 16 below shows:

- The number of operational sites in each parish that provided answers to the size estimation questions (total 415 sites)
The sum of the number of street sex workers reported in each district (total 701)
- The sum of the number of exotic dancers reported in each district (total 983)
- The sum of the number of other female workers at the venues, a group that often includes some female sex workers but are not as easy to identify and count as exotic dancers or street sex workers (total 647).

Table 16 Number of Workers at Venues

Region	Parish	Number of verified	Street SWs on a	Dancers on a busy night	Other Venue Workers

		sites with PSE data	busy night		
South-East	KSA	45	49	71	16
	St. Catherine	29	130	84	49
	St. Thomas	22	27	72	15
	<i>Regional Total</i>	96	206	227	80
Southern	Manchester	32	49	137	17
	St. Elizabeth	30	20	114	53
	Clarendon	20	82	98	8
	<i>Regional Total</i>	82	151	349	78
North- East	St. Ann	54	120	127	255
	St. Mary	41	0	25	59
	Portland	53	27	47	41
	<i>Regional Total</i>	148	147	199	355
Western	Trelawny	18	19	57	15
	St. James	18	78	68	72
	Westmoreland	12	2	22	11
	Hanover	40	98	61	36
	<i>Regional Total</i>	89	197	208	134
TOTAL		415	701	983	647
South East	SERHA Total	129	105	50	89
Southern	SRHA Total	109	136	185	130
North East	NERHA Total	174	249	307	567
Western	WRHA Total	89	158	101	134
	TOTAL SITES	501	648	643	920

Estimates from the PLACE Survey

Table 17 below shows the number of FSW interviewed in each region and the number of sites where these interviews occurred. Overall, there were an estimated 1303 sex workers at the venues where PLACE interviews were conducted.

Table 17: Estimated Number of Sex Workers at Survey Venues

Region	South East	North East	Western	Southern	Total
Number FSW Interviewed	331	201	231	257	1020
Estimated Percentage of FSW at Site that were interviewed	80%	62%	79%	91%	78%
Total Number of FSW Estimated at Sites Where Interviews Conducted	415	322	294	284	1303

Weighting was also used in the size estimates. Table 18 provides insight into how the weights were applied and shows the cross-sectional estimates for a busy night, for one month and 6 months. The estimate for 6 months is based on upweighting the one month estimate by 20.9%. This is the estimate of the proportion of the FSW population who will enter sex work over the course of 6 months. This proportion is based on questions asked in the survey regarding the length of time in sex work. Each month, approximately 3.5% of the sex worker population is new, based on responses during the survey.

Table 18: Cross-Sectional Estimates of the Number of Female Sex Workers from Survey Data

	Number Interviewed	Average Weight for Busy Night Estimate	Estimated Number of FSW at all Venues on A Busy Night	Average Weight for One Month Estimate	Estimated Number of FSW at all Venues on A Busy Night Over One Month	New FSW Over 6 Months	Estimated Unique FSW Over 6 Months
	A	B*	A*B**	C***	A*C	D****	
South East	331	2.5	823	8.2	2730	570	3300
North East	201	3.3	659	14.1	2840	593	3433
Western	231	3.1	710	12.3	2850	595	3445
Southern	257	1.7	425	11.7	3013	629	3642
Total	1020	2.6	2617	11.2	11433		13820

* Average Factor for Upweighting to Adjust for Not Visiting All Sites and Not Interviewing All Women at Sites

** Estimated Number After Upweighting (column A * Column B)

***Average Factor for Upweighting to Adjust for Not Visiting All Sites and Not Interviewing All Women at Sites AND also to adjust for people not Visiting every night

**** Number of New Women in Sex Work Over the course of 6 months

Table 19 below shows the size estimates by region and nationally, along with confidence intervals. The size estimates should be rounded to the nearest 100 when used for program planning to emphasize that these estimates are based on extrapolations which are not precise.

Table 19: Number of Female Sex Workers and Confidence Intervals

REGION	Sample	Busy Night Estimate	95% CI		Month Estimate	95% CI		6 Month Estimate	95% CI	
Southeast	331	823	562	1085	2730	1365	4095	3300	1650	4951
Northeast	201	659	351	966	2840	944	4736	3433	1141	5725
Western	231	710	379	1040	2850	1652	4047	3445	1997	4892
Southern	257	425	288	563	3013	1809	4218	3642	2186	5099
Total	1020	2617	2165	3069	11,433	8,852	14,014	13,820	10,701	16,940

STRENGTHS AND LIMITATIONS

Strengths of the Study

- Data collection using tablets was applied in Phase three, the survey phase. This eliminated the need for a separate data entry component. Additionally, the design of the questionnaire applied prompts and skips which minimised interviewer errors.
- The use of sound prompts on the tablets allowed participants to self-complete sensitive questions without having to give answers to the interviewer even when they had challenges reading the questionnaire. The interviewer read the questions from a separate form but was able to tell from the sound prompt when the participant was ready for the next question without looking at the participant's answer
- The programme staff's familiarity with the local /parish situations enhanced the implementation of all three phases of the study. In phases one and two staff members were able to select appropriate informants, locate and confirm or disconfirm sites named by community informants but also locate new emerging sites.
- In phase three (survey) the parish supervisors were programme staff leaders who were able to provide entry to the survey sites for the independent interviewers. This facilitated ease access to the venues for the interviewers while minimizing social desirability as participants were not being interviewed by programme staff whom they knew previously and who may have promoted behaviours being investigated in the study.
- The counting of different categories of participants at sites was a useful innovation for probability sampling and estimating the size of the population of dancers, street sex workers, patrons and PLACE workers.
- The use of the dried blood spot sample collection for measuring viral loads of HIV positive participants enabled the analysis of the treatment cascade.
- The ethical aspects of the study were observed consistently. Both interviewers and sample collectors observed high standards of confidentiality and respect for

participants. Every effort was made to conduct pre and post- test counselling privately despite the challenges at the sites, this included negotiating with the managers or other gate keepers for the use of a private room or area inside or outside the venues for interviews and sample collection. There were no complaints of breaches of confidentiality or privacy from any of the 1660 participants.

- The refusal rate among sex workers and workers was extremely low.

Limitations

- Unfortunately, there were delays in data collection between phases two and three across parishes. In phase two where participants were being counted, movement of women who solicit across geographical areas had to be borne in mind when assigning regional and parish quotas for the survey phase. The national count in February 2024 over one weekend was used to facilitate recruitment targets.
- Some parishes in the Western region were under curfew because of high levels of violence. This resulted in very low levels of data collection in those areas.

DISCUSSION

In this section the objectives of the study will be discussed based on the data obtained in the three phases of the study.

The HIV Prevalence in this 2024 IBBS maintains the steady decline in the prevalence of HIV among sex workers, moving from 4.9% in 2008 to 4.1% in 2011, 2.8% in 2014 and 2% in 2017. In this study a weighted prevalence of 3.2% among participants who reported exchanging sex for cash in the last 6 months with a confidence interval of 1.6, 4.9 while the prevalence among workers is 2.2% with a confidence interval of 1.0, 3.3 and 2.4% [0.1, 4.8] of patrons is not different from the 2017 IBBS that recorded a prevalence of 2% from 16 HIV positive participants of the 805 who reported ever exchanging for cash for sex. In the 2017 survey 748 participants reported exchanging cash for sex in the last six months and 578 reported sex work as their main source of income. Using any of these classifications the HIV prevalence lies within the 2024 confidence interval of 1.6, 4.9. Of note no female patron tested positive in 2017.

Syphilis testing indicated a low prevalence of 3.5% for the 2024 sample with 4.2% for workers (n=1185) and 1.4% (475) among patrons and 4.2% for women reporting recent sex

work. Based on the type of test used in the survey it cannot be determined without confirmatory testing how many are active cases. These results are like results from the 2017 survey. Further analysis using programme data should provide more detailed results.

It should be noted that the sex workers reached and tested in this survey could exclude certain types or categories of sex workers who are not captured in the radar both of this study and general targeted interventions, namely those who solicit remotely via telephone and social media. Additionally, qualitative research indicates that there are sex workers who solicit from other sites including the community or do so seasonally. These women are more likely to provide services without a condom or not be tested for HIV and other STIs. It is important that regular outreach for sex workers seek to identify and target these women as well as those in the traditional sites included in this survey.

There are several results in this study that indicate the vulnerability and risk of HIV and STI transmission. There are specific behaviours and experiences that create an increased risk of HIV infection among vulnerable populations. Among sex workers in this study failure to use condom with a main partner was way higher than failure to use condoms with a paying client. Exposure to violence, having spent a night in jail and the use of substances was highest among sex workers.

The main risk behaviour noted in this survey was the inconsistent use of condoms with main partners and regular paying clients. In this study 37% of workers, 59% of patrons and 36% of current sex workers reported no condom use with main partners. Ways must be explored to reach the regular client and main partners of sex workers as well as to continue to emphasize this aspect in the risk reduction conversations with the FSWs.

Incomplete education and early childbearing were the main socio-economic drivers and underlying determinants of entry into sex work and increased risk of HIV/STI infection. It is of note that the median age of sexual initiation (16 yrs.) of the participants was like that recorded for in the national reproductive health survey (10). The vulnerability created by poverty and lack of social support was also manifested in the adverse events recorded among participants as 68.4% of workers, 49.9% of patrons and 66.7% of sex workers had experienced violence. Additionally, at least a third of workers and sex workers had spent a night in jail and almost one in five has experienced homelessness. Interventions to reduce socio-economic vulnerability among FSWs should be strengthened and may require partnerships and structural approaches that go beyond the remit of the HIV/STI Programme and the Ministry of Health.

The participants particularly the FSW ways of coping with the physical and psychological conditions of sex work is manifested in the recorded common use of alcohol and drugs (<60% alcohol and marijuana use among sex workers) which in themselves create risk through disinhibition. Discussions with sex workers about the ways in which ingestion of alcohol and performance enhancing substances can be reduced to mitigate risk must be included in the interventions delivered to this target population.

Programme coverage was reflected in the high levels of exposure to HIV prevention messages and materials recorded by participants. The main messages promoted by the HIV/STI Programme and partner NGOs such as JASL through face-to-face interventions, print, radio advertising and videos on television and social media platforms include:

- Seek HIV and syphilis testing including self-testing e.g. *I do my HIV test I know my status I'm taking no more risks*; *Be in the know, get tested*; *Dweet (do it) for the best life, don't put yourself at risk*.
- Use condoms correctly and consistently e.g. *Get, carry and use condoms* or *Pinch, leave an inch and roll*.
- Seek treatment if you tested HIV Positive and use antiretrovirals consistently e.g. *Antiretrovirals for life* or *Feel better, live longer, take your meds, act responsibly*
- PrEP is available (promoted mainly among high- risk groups) e.g. *Put some PrEP in your step* or *Get on PrEP*

However, it is important to note that while 60% of workers and 66.6% of current sex workers had participated in condom demonstration only 37% of patrons had. Similarly, while over 60% of workers and sex workers had recent interaction with an HIV/STI prevention outreach worker only 32% of patrons had done so. Since some patrons at sites where sex workers operate also exchange sex for money, the patrons need to receive similar interventions as the sex workers.

The sex work population in Jamaica is dynamic and anecdotal evidence indicates a post COVID 19 increasing shift to online solicitation which can make female sex workers less visible. The population size estimates presented align well with formative research implemented as part of the study, including a national count of dancers, street sex workers, female workers and patrons. Although a fourth reported soliciting online weekly or more, 65% reported never soliciting online. Among the 41 who were recruited via coupon, 65% reported never soliciting online. We did not find evidence that there is a large group of women who only solicit online, however, this issue should be explored further.

14. RECOMMENDATIONS

- Constant surveillance is necessary to keep track of changes in HIV prevalence. Incidence cannot be measured in these types of cross-sectional studies. Incidence data provides evidence of where and how the infection is spreading and can inform targeted prevention and treatment efforts.
- Patrons (9.1%) and workers (15.6%) who are associated with the sites where sex workers and exotic dancers solicit admitted to accepting cash for sex (ever). This indicates they are also involved in transactional sex. Similar levels of intervention should be provided to these women.
- Ways must be found to adequately address the threat of partner intimacy to condom use. It is possible to reach the regular client and main partners of sex workers at some venues to have them assess their own risk of HIV/STI infection. This is necessary as the women are more likely to use condoms with these partners if the men are open to or request condom use (Watson, et.al. 2021; Woodsong & Koo., 1999)
- The use of alcohol and drugs must be addressed consistently particularly the use of ecstasy among exotic dancers and crack cocaine among street sex workers. The use of substances increase the risk of infection with HIV and other STIs (Sema, 2024; Beksinska, 2023).
- Programme staff need to identify strategies for reaching the less visible sex workers including those who solicit online . These are likely to be vulnerable females with less exposure to prevention interventions and messages. The data on these women is also needed for programme planning.
- We analysed survey data to estimate the size of the FSW population in Jamaica. The size estimates reported should be rounded to the nearest 100 when used for program planning to emphasize that these estimates are based on extrapolations which are not precise. These estimates could be improved by leveraging information provided by routine Programme data, stakeholder consultation, and review of previous estimates.

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APPENDIX 1

PROTOCOL

TITLE: Priorities of Local AIDS Control Effort (PLACE) Workers and Patrons including Men who have sex with men and Transgender women Survey

DATE: June 30. 2023 Revised October 27-2023

INVESTIGATORS:

CO-PRINCIPAL INVESTIGATORS: Dr Nicola Skyers, Senior Medical Officer, HIV/STI/TB Unit, Health Promotion & Protection Branch. Ministry of Health & Wellness and Wellness
Email: nicola.skyers@moh.gov.jm, Telephone: 876- 633-8214

Mrs. Marion Scott Thomas. Adolescent Programme Manager, National HIV/STI/Tb Unit, Ministry of Health & Wellness. 10-16 Grenada Way, Kgn. 5, Jamaica. Contact information, Email: marion.scott@moh.gov.jm and Telephone: 876-633-8172

CO-INVESTIGATORS: Dr Noel Watson, CEO of A-Z Information Jamaica Limited the Ec-biz Centre, 2 Trafalgar Road, Kingston 5. Email: info@azinfojam.com, noelwatson@azinfojam.com. Tel: (876) 997-9489; 1(876) 927-6137

Dr Althea Bailey Senior Lecturer University of the West Indies Mona Kingston. 7 Telephone 876-816-3679

SITE OF RESEARCH: Cross-sectional bio-behavioural surveys of people at venues identified as places where people meet new sexual partner in order to estimate prevalence of infection among this population, risk factors associated with infection and gaps in STI prevention programs.

NUMBER OF PARTICIPANTS TO BE ENROLLED: Approximately 3,880 individuals will be interviewed in a minimum of two study areas. These individuals will be community informants, venue informants, PLACE workers (PW), and PLACE patrons including MSM and TG. Approximately 1,140 individuals will be interviewed from the Southeast health region, 875 from the Western region and another 875 from both the Northeast and Southern regions. Additionally, 500 MSM & TG from all regions (See table 1) will be interviewed. The target sample size will provide sufficient power in data analysis to identify and describe the proximate factors associated with risky sexual behaviour and HIV/STIs prevalence in the sample of participants. The allocation of FSW and FP interviews across areas is based on the expected size of the FSW and FP populations in each of the regions. The allocation of MSM and TG is based on the expected reachable members of this mainly hidden population.

Table 2: Proposed distribution of participants by regions

Participants	Priority Prevention Areas (PPAs)	Gender	Age
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	South East	North East/ Southern	Western	Total		
Community Informants	400	400	400	1200	M&F	18+
Venue Informants (1 per venue)	340	170	170	680	M&F	18+
Female Sex Workers (FSW)	250	175	175	600	F	16+
Other Female Patrons (FP)	80	70	70	210	F	16+
Other Female PLACE Workers (FPW)	70	60	60	190	F	16+
Male Patrons (MP) and Male PLACE Workers (MPW) at MSM Venues and Events	500	250	250	1000	M	16+
Total	1640	875	875	3880		

PROPOSED DURATION OF THE STUDY: The study will take approximately eight (8) months to complete. The data collection component entails three sets of interviews, they are: A. community informant, B. site verification and C. interviews with workers and patrons. The first phase of data collection will commence with community informants. Approximately 5-10 minutes will be scheduled per interview. Site verification interviews will follow community interviews and about 20 minutes per interview is expected. The final set of interviews will take place with sex workers, female workers, female patrons, transgender women and men who have sex with men. The estimated time per interview in the final stage of data collection will be about 30-40 minutes.

SUMMARY

All aspects of the 2011, 2014 and 2017 bio-behavioural surveillance survey on CSW and patrons were approved by the Ethics Committee. Regarding female participants, the current protocol reflects only minor changes to the 2017 PLACE survey, in line with the current focus on female sex workers, patrons and staff. This present focus is in response to the data that emerged from the previous survey, among other factors, which showed female sex workers and female workers to be at an increased risk, approximately 10-11 more likely, of being diagnosed with HIV when compared to male patrons. This pattern was consistent when data for Syphilis was examined across the different groups mentioned above (Ministry of Health & Wellness & Wellness, 2012).

HIV prevalence rates among MSM in Jamaica are high with an estimated HIV prevalence of 31.4% in 2011 (Figueroa et al 2017) and 29.6% in 2017 (876 survey 2017). Ten percent (10%) of all new HIV infections reported in 2018 were among men who have sex with men (MSM). Local studies in 2011 and 2017 found an approximately 50% prevalence of HIV among transgender women, which is the highest prevalence of any population group.

The aim of this protocol is to describe the PLACE approach that will guide the execution of the 2023 bio-behavioral surveillance survey (BBSS) among FSW, other FPW, FP, MPW, MSM and TGW at sites that they frequent. The survey will establish HIV prevalence among these groups and collect information that will guide and improve HIV/STIs interventions among these populations.

Specific objectives are:

- To determine prevalence and incidence of HIV and Syphilis among high-risk women, high-risk men, and transgender women (TGW)
- To measure proximate & underlying determinants of HIV and STI transmission
- To monitor program coverage
- To establish closer contacts with key populations in order to expand prevention and treatment services.
- To determine effectiveness of prevention interventions among FSW, MSM and TGW

- To estimate female sex workers population size
- To understand social and sexual networks of FSW

A moderate risk to participants of emotional distress exists due to the nature of the study. A safe and supportive environment including referral for no cost services will be made available to those who need it. No identifying information will be collected from participants in relation to the behavioural components of the study. The guidelines and procedures that will be followed in the 2023 PLACE Survey for conducting interviews with persons at increased risks of acquiring and transmitting infections as well as HIV and Syphilis testing, pre and post-test counselling and provision of test results to participants and ensuring confidentiality and quality control are the same as those that were approved in the 2011, 2014 and 2017 PLACE Survey. These guidelines and procedures have been further refined and published in *PLACE in the Caribbean; Institutionalizing PLACE methodology to reach key populations in the Caribbean* (Scott & Byfield, 2013).

BACKGROUND

Jamaica has both a generalized and a concentrated HIV epidemic. In 2021 HIV prevalence among adults was 1.3% with an estimated 30,000 persons living with HIV (PLHIV) (UNAIDS Country Fact Sheet - Jamaica 2021). The Ministry of Health and Wellness HIV estimates for Jamaica states that as many as 5,293 Jamaicans may be infected with HIV and they are unaware of their HIV status (UNAIDS, 2019). Approximately 27,324 persons have been diagnosed with HIV and most of them were linked to medical care. However, 14,069 of these persons have been lost to care and may not be on treatment. As of December 2021, 14,000 (47%) PLHIV were on anti-retroviral treatment. An estimated 80% of these persons were virally suppressed, which is an indicator of successful treatment. These estimates are based on UNAIDS models using data from the Ministry of Health including the HIV surveillance system and a variety of surveys conducted in Jamaica.

The Ministry of Health and Wellness HIV/STI/TB Report 2019 states that among persons reported as newly diagnosed HIV cases in 2018, 63% lived in Kingston and St Andrew, St James or St Catherine. More than 50% of MSM report also having sex with women. 43% of all newly diagnosed HIV infections in 2018 were among heterosexual men with a high-risk profile – multiple sex partners, having a sexually transmitted infection (STI) or being a client of a sex worker. Data on high-risk populations reveal that 13% of new HIV infections were among women of high risk and 2% among female sex workers. 10% of all new HIV infections reported in 2018 were among men who have sex with men (Figueroa et al 2017, Ministry of Health and Wellness 2017).

There has been much progress made in reducing HIV prevalence among female sex workers in Jamaica. HIV prevalence among female sex workers was estimated to be 12% in 1990, 9% in 1997 (Hope Enterprises 2005), and 2005 (Duncan et al 2010), 4.9% in 2008 (Duncan 2014), 5.4% in 2011 (Weir et al 2018) and 3.2% in 2014. A recent survey in 2017 estimated a HIV prevalence of 2% (Survey of Commercial Sex Workers 2017). However, caution should be exercised in interpreting this since it could be that sex workers who knew they were HIV positive failed to participate in the survey. HIV prevalence is twice as high among female street sex workers compared with other sex workers. While reported condom use is very high among female sex workers with their clients, it is known that a condom is not always used with a regular client and frequently not used with the main partner of sex workers (Bailey & Figueroa 2017). The high prevalence of other STI among sex workers is consistent with the failure to use the condom at all times.

The Jamaica National Strategic Plan 2020-2025 lists the factors driving the HIV epidemic in Jamaica including but not limited to the following:

- The high-risk behaviour of heterosexual men including unprotected sex with commercial sex workers, multiple sex partners and transactional sex.
- The very high HIV prevalence among MSM due to unprotected anal intercourse with multiple sex partners.
- The high rate of bisexuality among MSM that acts as a bridge for HIV transmission into the general population.
- The high rates of other sexually transmitted infections.
- The failure to affirm the rights of MSM, sex workers and other marginalized groups and decisively combat the stigma and discrimination associated with HIV and homosexuality.

There are many social and cultural factors that contribute to the spread of HIV (Figueroa 2009). Sex begins at an early age. The median age of first sex for boys has decreased from 16.5 years in 1996 to 15 years in 2017, and for girls from 18.2 years in 1996 to 17 years in 2017. As many as 42% of boys and 12% of girls aged 15 – 24 years reported that they initiated sex before they were 15 years of age. If one is poor and unskilled the options for getting work are limited. Many females will therefore turn to transactional sex in order to survive or do better socially. Transactional sex was reported by 45.4% of men and 16.5% of women in 2017 compared with 53.8% of men and 23.6% of women in 2012 (Skyers et al 2017. Hope Caribbean Co. Ltd 2017). Transactional sex may be more prevalent than these figures suggest especially among women of lower socio-economic status. If things become desperate some women will turn to commercial sex, which has grown considerably in the past few decades and takes a variety of forms (Bailey & Figueroa, 2017). These realities will likely exacerbate the negative impact of general socio-economic challenges on the population of sex workers and further increase risky sexual practices. The current 2023 bio-behavioural surveillance data is critical in informing policies and programs to better respond to the sexual health risks of sex workers, MSM and Transgender women.

Research Questions

The study will explore the following questions:

- What are the characteristics of venues where people meet new sexual partners in Jamaica?
- Where are venues located?
- What evidence is there of HIV prevention program efforts at the venues?
- Are condoms available onsite?
- Are HIV prevention messages displayed onsite?
- What is the prevalence and incidence of HIV and STIs among men and women at venues identified as places where people meet new sexual partners?
- What is the prevalence and incidence of HIV and STIs among women who exchange sex for money, transgender women (TGW), and men who have sex with men (MSM)?
- What are the socio-demographic and behavioural characteristics of these groups?
- What is the extent to which these groups have access to HIV prevention programs?
- What are the characteristics of those infected with HIV?
- What are the characteristics of those infected with other STIs?

Rationale

There is an urgent need in Jamaica to improve current prevention programs to limit the spread of HIV. Many approaches are needed to contain the epidemic including school-based programs, mass media campaigns and targeted approaches for those most at risk. Prevention efforts require current information about the HIV and STIs epidemics in high-risk groups.

Participants:

- 1200 Community Informants aged 18 and older will be asked to identify venues.
- 680 Venue Informants will be asked about the characteristics of a venue that he or she is knowledgeable about (1 informant per venue)
- 600 Female Sex workers (FSW)
- 400 'Other female PLACE' workers (FPW) and Female patrons (FP)
- 500 Men who have sex with men and TGW
- 500 'Other' high risk male PLACE workers (MPW) and patrons (MP).

Participants will be recruited from the four health regions. The sample size was calculated in order to have separate precise estimates categories of participants (i.e., FSW Other PW and FP) and to have a large enough sample to allow comparison with previous surveys. The number of community informants is based on the expectation that 400 interviews in each area will identify over 90% of the eligible venues. This reflects experience with previous surveys in Jamaica.

Inclusion criteria and how identified and recruited:

- Community Informants: 1200 Adults ages 18 and older who are expected to be knowledgeable about where people go to meet new sexual partners. Community informants will be recruited by convenience from settings where target population members are likely to congregate or visit.
- All females at selected venues including all female workers and patrons will be recruited by study team members, assessed for eligibility, invited to participate using an informed consent process (described later). Those who consent will be interviewed and tested. Recruitment is a "take all" approach. Identification of patron, worker and sex worker status will be based on responses to the interviews. We expect to interview and test:
- FSW: Women who report exchanging sex for cash money in the past 12 months (n=600). Other Female PLACE Workers (FPW): Women who (n=190) work at a venue identified by community informants as a place where people go to meet new sexual partners. Female Patrons (FP): Female patrons (n=210) at PLACE venues. If the number of female patrons exceeds the number that can feasibly be interviewed, they will be recruited by study team members using a process that obtains a representative sample of patrons from the venue. The method will be adapted to fit the type of venue. For example, in closed environments such as a bar or club, an imaginary X

will be drawn by the study team across the floor of the venue and persons standing near the X will be approached for an interview. This approach has been implemented in previous patron surveys including the 2011 PLACE Survey.

- Male Patrons (MP) and Male PLACE Workers (MPW): All males (n=1000) at venues identified as places where MSM meet new sexual partners will be screened. During the screening, if the person does not meet the eligibility criteria or if targets for that type of participants have already been reached, the interview will be concluded. The Screening Questionnaire is Form C and it includes obtaining informed consent.
- Transgender Persons: All transgender persons will be interviewed.
- Persons ages 16 or 17 and at the venue as an adult, that is, they are not with a family member at the venue nor on a family errand are eligible to participate. It is noted that the mean age at first sex in Jamaica is 14.7 years (Jarret et.al.2018; KAPB 2017). High percentages of sexually active youth reported engaging in risk behaviours including transactional sex (43.0%) and 24% of females ages 15-24 engage in transactional sex (KAPB 2017). It is therefore possible and also likely that males and females 16 and 17 years old, at sites where persons go to meet new sex partners are engaged in transactional sex.
- Exclusion criteria:
 - Persons younger than age 18, unless the person is age 16 or age 17 and at the venue as an adult, that is, they are not with a family member at the venue nor on a family errand, will be precluded from participating. This criterion was used in previous PLACE surveys

Study Design:

Cross-sectional bio-behavioural surveys of men and women at venues identified as places where people meet new sexual partners in order to estimate prevalence of infection among this population, risk factors associated with infection and gaps in STIs prevention programs.

Study Procedures

- Stakeholder Meetings: One stakeholder meeting in each of the 4 health regions will be convened to explain the purpose of the surveillance to the community, identify types of people who could be approached as community informants, specify target numbers of each type to interview, and generate a preliminary list of venues. Stakeholders will include peer educators and outreach workers.
- Community Informant Interviews: Following this meeting, brief interviews with 1200 community informants will be conducted. Interviewers will ask each informant a few questions about himself or herself (See questions in appendices under the subsection Form A) and to name venues where people meet new sexual partners and where each venue is located. No personal identifiers will be obtained from the informant. Community informants in previous surveys in Jamaica have included taxi drivers, people who work in bars and clubs, peer educators, and others. The list of venues named and their addresses will be compiled in an Excel file and sorted by geographic area. The number of informants reporting each venue will be tallied and the list collapsed into a comprehensive list of unique venues.
- Venue Visits/ Site Verification: Trained interviewers will visit each reported venue and event (up to a maximum of 680 venues) and interview a respondent at each venue (Form B) about the characteristics of the venue. Form B will include questions about whether there are workers and patrons at the venue and how many (if any) of the workers and patrons are likely to be FSWs or MSM. The respondent may be a manager, a worker, or someone else who is knowledgeable about the venue. No (personally) identifying information will be obtained from the respondent. GPS coordinates will be recorded for each venue. Data from Form B will be entered and tabulated.
- Sampling Venues and Events for Interviews with patrons, and other PLACE workers: The study team will review the venue data from Form B to determine which venues and events to visit for bio-behavioural interviews based on statistical sampling methods as well as pragmatic considerations such as how many people of the target group are likely to be reached at the venue and the likely willingness of the people to participate. The criteria for

inclusion in the survey is as follows: all sites with sex workers or MSM present at the busiest time, 10% of sites with 25-50 females but no known sex worker at the busiest time and 10% of sites with less than 25 females and no known sex workers at the busiest times.

- Questions on Form C will be posed to these participants. The Study Lead will use the existing interventions/workshops and outreach activities to sensitize and to encourage participation. The recruitment process, experiences and questionnaires used at venues will be monitored and documented by supervisors using Form E. The survey and testing will be conducted at times arranged with the participants at the selected venues or locations identified as comfortable and safe.
- Screening for Recruitment into sex worker, other PLACE worker and patrons: There are no specific criteria that must be met in order to participate in the initial screening. During the screening, if the person does not meet the eligibility criteria or if targets for that type of participants have already been reached, the interview will be concluded. The Screening Questionnaire is Form C and it includes obtaining informed consent.
- Eligible Persons Invited to participate in survey and testing: Eligible persons will be invited to participate in the survey and testing. Only persons who have agreed to survey and testing for HIV and Syphilis. At the end of the interview telephone data worth JD\$1000 will be given to participants. Participants coming to events may be given \$2000 to cover the cost of transportation.
- Informed Consent: People will be asked to give verbal consent once they are identified as eligible to participate in the survey. All participants will be asked to consent to both the interview and testing. Venue workers and patrons include male venue workers and patrons. Individuals will be advised of their right to refuse to participate or to end participation at any time.
- Content of Surveys: The survey will administer three separate questionnaires (Forms A-C), plus an additional confidential answer sheet (Form D) for sensitive questions posed on Form C. Form A will be administered to Community informants and will contain questions on their age, occupation, what parish they reside in and the name(s) and location of places that they know of where SW solicit clients, and where people go to meet new sexual partners.
- Form B will be used in the site verification and will be administered to a knowledgeable person at each of the venues selected for verification. The instrument will capture data about the number of people attending the venue, their characteristics, and about the environment of the venue (e.g. is alcohol served).
- The questions on Form C will be administered to sex workers, male and female patrons and staff and will include questions about sexual behaviour, signs of STI, exposure to HIV prevention programs, and knowledge of HIV. Responses for the most sensitive questions about sexual behaviour and condom use will be documented by participants themselves on a confidential card (Form D). Form D will be separated from the rest of the instrument and be given to the participants to complete themselves by circling or ticking their response and placing that in an envelope without the interviewer seeing their response. The questions will be read to them to ensure low literate/illiterate participants will be able to provide answers. This has proven to address participants' concern of confidentiality, desire for privacy and promote validity of responses.
- Each of these questionnaires (A-D) had been approved by the ethical review committee at the Ministry of Health & Wellness for previous surveillance studies, including the 2011, 2014 and 2017 PLACE surveys.

9. Rapid Testing and Provision of Results: Both rapid HIV and Syphilis tests will be conducted at sites by trained practitioners. The procedures which will be used to conduct the tests will adhere to the outreach testing algorithm protocols for HIV and Syphilis approved by the National Public Health Lab (NPHL). The Standard SD Bioline 3.0 rapid method will be used to test for Syphilis. All labels will use a unique identifier code generated from participants' information (comprising of part of the participant's birth date and mother's last name). Each of the tests will be accompanied by the required pre and post-test counselling, informed oral consent, standard safety and operating procedures and quality control measures. Counselling will be conducted in enclosed and private areas.

Persons whose test results are negative will be provided with risk reduction counselling. Those, whose results are reactive or inconclusive, in accordance with the NPHL protocols, will be counselled and referred to the Contact Investigator at the nearest public Health Facility or a private clinician of their choice for confirmatory testing

including CD4 count and anti-retroviral treatment. Class 1 notification form will be completed and delivered to the Parish Medical Officer of Health at the Parish Health Department for persons who tested positive for HIV, and Syphilis.

Persons whose test results are negative will be provided with risk reduction counselling. Those, whose results are reactive or inconclusive, in accordance with the NPHL protocols, will be counselled and referred to the Contact Investigator at the nearest public Health Facility or a private clinician of their choice for confirmatory testing including CD4 count and anti-retroviral treatment. Class 1 notification form will be completed and delivered to the Parish Medical Officer of Health at the Parish Health Department for persons who tested positive for HIV, and/ or and Syphilis.

10. Methods of recruiting: All participants will be recruited by experienced, well-supervised interviewers.

- a. Community Informants: Community informants will be recruited by trained interviewers from public locations in each study area in Jamaica. Interviewers will be provided with a list of individuals to interview with target numbers and types (e.g. taxi drivers, security guards, market sellers) by the Study Lead. Potential community informants will be approached by a trained interviewer who explains the purpose of the study and requests verbal informed consent. The interview and request for informed consent will be conducted in English and Patois, as appropriate. The respondent's name or other identifying information will not be asked or recorded.
- b. Knowledgeable persons at venues: Interviewers will visit the venues reported by community informants to verify existence and location and to interview a person knowledgeable about the venue, such as a manager or owner, to obtain characteristics of the venue and of the patrons who socialize at it. The questions do not require specific knowledge of the business operations at the venue. If no one is available for interview on the first visit, the venue will be re-visited. The criteria for identifying a suitable respondent are broad to prevent deductive disclosure of identity and to limit the number of re-visits. Verbal consent for an anonymous interview will be obtained for each completed interview. The interview and request for informed consent will be conducted in English and Patois, as appropriate.
 - i. Coupons will be given to knowledgeable persons at venues such as a manager or worker to distribute to females or MSM who regularly visit/ sell sex out of that site to be presented to study staff when data collection for the survey occurs in order to facilitate the size estimation of the population of sex workers. All males and females at the venues on the dates of the survey will also be counted.
- a. Patrons, workers, and sex workers at socializing venues: All participants will be recruited by experienced, well-supervised interviewers. Interviewers will approach potential study participants socializing at the study venues, explain the study, and request verbal informed consent. The interview and request for informed consent will be conducted in English and Patois, as appropriate. All participation is voluntary. If anyone cannot understand the purpose of the study, they will not be asked to give informed consent nor will they be asked any of the study questions.
 - i. Females socializing at the venues will be offered an incentive of JM\$1500 to recruit 3-5 other females who solicit via the telephone or social media who the study team will be unlikely to reach at the venues. They will be asked to take/ invite these females to visit the venue or a neutral location on the next data collection date. The incentive will be paid when the invited females turn up to participate in the survey. these invitees will go through the same information and consent process as other females that are recruited by the study staff. Similarly, MSM and TG at the venues/events will be offered the same incentive to recruit 3-5 other MSM and TG. They will be asked to invite these MSM/TG to the venue or another neutral location on the next data collection date. Coupons will be numbered. Female coupons will be a different colour than MSM/TG coupons.

11. Data confidentiality and quality control:

Electronic data capture using forms C&D will be used to record the survey responses, identify lab specimens and record test results. Interviewers will enter data using secure password protected tablets and will also be provided with paper form as back-up in case of loss of power. Participants will be asked to complete the confidential component in form D privately by circling or ticking answers on a paper form. The questions will be read to them by

the interviewer who will not see the answers completed by the participant. Form D will be placed in an envelope and sealed. The data from form D will be entered by the data entry staff.

Each form will be identified with a study identified ID number (comprising of part of the participant's birth date and mother's last name) that can link the forms. The only place that a participant's ID number and the name will be stored together is in a Study Log. The log will be kept under lock and key under the care of the Study Lead, with the two Principal Investigators being the only other officers having access to the safe. No names will be used in the analysis or reporting of the data. Persons testing positive for HIV and Syphilis will be notified under confidential cover in keeping with present practice under the Public Health Act.

12. Data analysis and Reporting: Data will be entered in SPSS form F (as shown in appendix) has been created to track and monitor the data processing process. Tables will be generated in SPSS.

13. Benefits to subjects and/or society: Benefits to the participants include getting tested for HIV and Syphilis as well as referral for treatment where required. Individuals at the venues may benefit from the programs if they are still in the community when the programs are implemented. There are potential benefits to society as results will be used by stakeholders such as the National HIV/STI Program of the Ministry of Health & Wellness and the NFPB-SHA to design HIV prevention interventions for sex workers, high risk women, high risk men, and for the general population. The key persons at the Ministry of Health & Wellness and NFPB-SHA who will use the data are the Senior Medical Officer Dr Nicola Skyers, HIV/STI/TB unit, Mrs Marion Scott-Thomas, Adolescent Programme Manager and Mrs. Rahanah Khan- Francis, Strategic Information Coordinator, HIV/STI/ TB to improve programs in the area of HIV and STIs prevention and control.

14. Full description of risks and measures to minimize risks: A moderate risk to participants of emotional distress exists due to the nature of the study. A safe and supportive environment including referral for no cost services will be made available to those who need it. Participants who report rape or physical abuse will be referred to The Centre of Investigation for Sexual Offence and Child Abuse (CISOCA)

There is minimal risk of breach of confidentiality from this study. Confidentiality of subjects will be maintained and protected by the use of study numbers on study specimens, case report forms and other study forms. Study documents including consent forms (Form I in appendix) with the subjects' names, will be stored in a separate logbook in a locked cabinet at the NFPB-SHA. Syphilis and HIV are reportable diseases in Jamaica. All HIV and Syphilis results will be entered into the MOH surveillance system using the standard system to maintain confidentiality of patient results. To ensure confidentiality and minimize risks to participants, all HIV, Syphilis and STI testing will be conducted by staff already employed to the National HIV/STI Programme or the Regional Health Authorities who conduct outreach VCT for HIV and Syphilis as part of their routine function.

Community informants will be asked to name public places where they believe persons visit to meet new sexual partners. Providing this information may cause feelings of embarrassment and the study provides persons with the option of refusing to participate. No one can be put at risk of legal prosecution because no identifying information will be recorded.

Knowledgeable people asked about the venues may feel embarrassed to report that people meet new sexual partners at the venue, or that commercial sex workers seek clients at the venue. There is no reason for them to fear official retaliation against the venue because local owners and manager as well as local officials including the police will have endorsed the assessment. There have been no reprisals against sites in any of the over 30 completed implementations of PLACE assessments.

Participants may feel embarrassed to report their socio-demographic characteristics or sexual or other risky behaviour. However, protocols were developed to minimize the feelings of embarrassment or reluctance that a person may have in answering the questions. Participation is brief, voluntary and anonymous. Anyone may refuse to answer any question at any time. The purpose of sensitive questions is described to allay respondent's fears. The most sensitive questions will be asked using self-completed answer sheets. This proposed procedure has been used for previous surveys in Jamaica with approval from the Advisory Panel on Ethics and Medical Legal Affairs Team of the Ministry of Health & Wellness. No breaches of confidentiality have been previously reported.

15. Data monitoring and analysis: Approximately 3,880 individuals will be interviewed in the following areas: community informant, site verification and participant interview (including sex workers, other PLACE workers, male and female patrons, including MSM and TGW). The target sample sizes will provide sufficient power in data analysis to describe the sexual behaviour of people with a high rate of new sexual partnerships. The data collected are quantitative and will be analyzed by generating frequencies and cross-tabulations for a final report. Data may be stratified by age (18-24, 25+), risk types, educational attainment, regions, among other variables.

16. Identifiers in research data: The only place that a participant's ID number and the name will be stored together is in a Study Log that is only available to the Co-Principal Investigators and the Study Lead. Information from the log will also be confidentially entered into the national surveillance database for persons infected with HIV or Syphilis. This process has been ongoing in Jamaica for over 30 years and there has been no known breach of confidentiality.

17. Confidentiality of the data. Confidentiality will be discussed in detail with field workers. Each field worker will be required to sign a pledge of confidentiality (See form G in appendix). Confidentiality issues will be addressed during a training session on research ethics. The PowerPoint Presentation used in ethics training for interviewers in previous PLACE studies will be used at the time of interviewer training.

Most of the venues are public venues where anyone is welcome to socialize. A few private venues and private events may sometimes be included in the study. The list of names and addresses of venues will not be shared with anyone outside of the study team.

The characteristics of venues obtained from knowledgeable people at the venues will not be shared outside of the study team except in reports summarizing characteristics across all venues. For example, the report might state that 'knowledgeable people at 50% of venues reported that beer is consumed onsite' but a list of venues by name where beer is consumed will not be provided. Similarly, characteristics of patrons socializing at these venues will be presented in summary form by age group and regions, and not by site.

Maps of the venues will be shared with the intervention team and published but not in a manner that would identify illegal activities or permit easy identification of venues. The large size of the dots also masks exactly where a venue is located. Without landmarks or the name of the neighbourhoods, it is difficult for those not on the study team to recognize the study area. The purpose of the mapping is to show clustering and the extent of risk behaviour, not to identify particular venues so that they can be located. Consequently, the base maps used to ground the points collected by GPS are invariably incomplete and distort the true location of venues.

Three types of maps are provided to the intervention team and also published in the report:

- (1) a map of the venues showing the number of times a community informant reported the venues
- (2) a map of the venues showing whether condoms and other AIDS prevention programs are available onsite or not
- (3) a map identifying priority venues for HIV/AIDS prevention programs. These maps may take into account the presence of commercial sex, patronage and alcohol consumption but would not be labelled in a way that would identify that any location was the venue for onsite illegal activity. Venues for commercial sex socializing would not be identified on any map in any report or publication.

Paper copies of completed questionnaires will be kept in a locked cabinet at the NFPB-SHA office. Only the Principal Investigators and the Study Lead will have access to the cabinet. The data will be stored on a password-secured hard drive of the data entry computer and saved on flash drives will be kept in the locked cabinet.

Data security for storage and transmission

Data security will be ensured by the use of software such as Redcap or Research Manager with access limited to the data analysis team Password protected/ encrypted files will be used to store data

Post-study disposition of identifiable data or human biological materials

No blood samples will be stored as part of the 2023 PLACE Protocol. The Study log will not be destroyed. After the study is completed and the results published, it will be kept by the Principal Investigators in a secure and locked place, awaiting use in a follow-up study. The soft copy of the data set will be owned by the M&E Units of the National Family Planning Board and the National HIV/STI Programme. Any related publication must be with permission of the Principal Investigators.

Informed Consent: The interviewer will request verbal, anonymous informed consent after the purpose of the study is briefly described. The interview and request for informed consent will be conducted in English and Patois.

Workers and patrons will be asked to sign an informed consent statement. They will be allowed to sign their initials if they are concerned about signing their name. They will be consent separately to interview and HIV testing.

Justification for a waiver of *written* (i.e., signed) consent: The only record linking the subject and the research would be the consent document and the principal risk would be potential harm resulting from a breach of confidentiality (e.g., study topic is sensitive so that public knowledge of participation could be damaging). *Participants will be asked whether they want documentation linking them with the research and the participants' wishes will govern whether they sign the form.*

Those who do not wish to sign the form can place an X, this must be witnessed by an independent person.

The only place that the names of the participants are recorded will be the study log. We do not want to have any other form with the name of the participants due to the potential harm resulting from a breach of confidentiality.

Persons whose HIV or Syphilis test results are found to be presumptively positive will be appropriately counselled and the necessary referral will be made for treatment including CD4 count and anti-retroviral treatment, where indicated. The log will be kept under lock and key under the care of the Study Lead with only the Principal Investigators having back up access. No names will be used in the analysis or reporting of the data.

This study involves a moderate risk of emotional distress to participants. The survey will pose questions to participants about sexual behaviour and condom use. A safe and supportive environment including referral for no cost services will be made available to those who need it. Participants will be required to provide a blood sample (a fingerpick for the HIV and syphilis test and no more than 5cc for those with a positive HIV rapid test to assess HIV viral load. The sample will be obtained using sterile lancet. A new lancet will be used for each of the participants. The bio-sample taking process will not adversely affect participants as all protocols for the taking of bio-samples in the public health system will be adhered to. Access to the names of participants will be restricted to only two officers who will be legally bound to adhere to the data confidentiality and management requirements of the survey. These officers will be the Study Lead and Principal Investigators.

Where will the subjects be studied? Communities within each health region of Jamaica will be identified for the study during the stakeholder meetings. Most interviews will be conducted in public locations in quiet, private areas within a public location. A few private venues may sometimes be included in the study.

Privacy: Prior to the implementation of the study, experienced interviewers will be trained on survey techniques and confidentiality issues associated with this study. They will be instructed in procedures for keeping all parts of the interview confidential.

For the participant, efforts will be made to ensure the interview takes place in a private setting for the purposes of confidentiality and to reduce any potential stigma. Interviews will, to the extent possible, be conducted in venues where people socialize, though in some cases the respondent may request to move to a different location within the venue in order to preserve privacy (e.g. a room set aside by the venue manager, an unoccupied corner of the venue, outside the venue, a private residence). Upon arrival at the venue, the interviewers will identify a suitable location where privacy can be maximized, checking to be sure the participants' responses cannot be overheard by others at the venue. If a suitable location cannot be found within the venue, interviewers will explore other locations outside and nearby the venue. All females present at sites may be considered for participation if they meet the eligibility criteria.

Incentive for participation: Study Participants will be given a telephone data with the value of up to JD\$1000 as an incentive for participation. Those who get results and are infected will be appropriately counselled and treated. Those who are HIV positive will be referred for appropriate management including CD4 count and anti-retroviral treatment where indicated. Where the referrals are to private doctors, the study will pay the initial visit costs. The main inducement to participate is to get tested for HIV and Syphilis.

Costs to be borne by subjects: The only cost to participants is time.

Use of Existing Data, Records or Human Biological Specimens: Previous surveillance studies conducted by the Ministry of Health & Wellness have included surveillance of the same study population. Information on test results from previous surveillance surveys has been collected. It is possible that some of the same people who were tested previously will be tested again. We will ask persons if they have been tested previously. If they have been tested previously, we will ask them for permission to look up their previous results in order to compare them with the new test results.

Funding: This study is being funded by the HIV/STI/TB Unit in the Health Promotion and Protection Branch of the MOHW with project funds received from Global Fund.

Declaration of Conformity with Guidelines: All Principles enunciated in the Ministry of Health & Wellness's Guidelines for the Conduct of Research on Human Subjects have been complied with.

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of the Department. I understand also that these provisions apply not only during the period of my employment but also after my employment in the public service has ceased.

Signed

Witness

Date

Extracts from the Official Secrets Acts, 1911 and 1920

Section 2 of the Official Secrets Act, 1911, as amended by the Official Secrets Act, 1920, provides as follows:—

"2(1)—If any person having in his possession or control any secret official code word or pass word or any sketch, plan, model, article, note, document, or information which relates to or is used in a prohibited place or anything in such a place, or which has been made or obtained in contravention of this Act or which has been entrusted in confidence to him by any person holding office under Her Majesty or which he has obtained or to which he has had access owing to his position as a person who holds or has held office under Her Majesty, or as a person who holds or has held a contract made on behalf of Her Majesty, or as a person who is or has been employed under a person who holds or has held such an office or contract:—

(a) communicates the code word, pass word, sketch, plan, model, article, note, document, or information to any person other than a person to whom he is authorized to communicate it, or a person to whom it is in the interests of the State his duty to communicate it; or

(aa) uses the information in his possession for the benefit of any foreign power or in any other manner prejudicial to the safety or interests of the State; or

(b) retains the sketch, plan, model, article, note or document in his possession or control when he has no right to retain it or when it is contrary to his duty to retain it or fails to comply with all directions issued by lawful authority with regard to the return or disposal thereof; or

(c) fails to take reasonable care of, or so conducts himself to endanger the safety of the sketch, plan, model, article, note, document, secret official code or pass word, or information;

that person shall be guilty of a misdemeanour.

(1A) If any person having in his possession or control any sketch, plan, model, article, note, document, or information which relates to munitions of war, communicates it directly or indirectly to any foreign power or in any other manner prejudicial to the safety or interests of the State, that person shall be guilty of a misdemeanour.

(2) If any person receives any secret official code word or pass word or sketch, plan, model, article, note, document, or information, knowing or having reasonable ground to believe, at the time when he receives it, that the code word, pass word, sketch, plan, model, article, note, document, or information is communicated to him in contravention of this Act, he shall be guilty of a misdemeanour, unless he proves that the communication to him of the code word, pass word, sketch, plan, model, article, note, document, or information was contrary to his desire."

Section 1 (2) of the Official Secrets Act, 1920, provides as follows:—

"(2) If any person—

(a) retains for any purpose prejudicial to the safety or interests of the State any official document, whether or not completed or issued for use, when he has no right to retain it, or when it is contrary to his duty to retain it, or fails to comply with any directions issued by any Government Department or any person authorized by such department with regard to the return or disposal thereof; or

(b) allows any other person to have possession of any official document issued for his use alone, or communicates any secret official code word or pass word so issued, or, without lawful authority or excuse, has in his possession any official document or secret official code word or pass word issued for the use of some person other than himself, or on obtaining possession of any official document by finding or otherwise, neglects or fails to restore it to the person or authority by whom or for whose use it was issued, or to a police constable; or

(c) without lawful authority or excuse, manufactures or sells, or has in his possession for sale any such die, seal or stamp as aforesaid,

he shall be guilty of a misdemeanour."

S.P.O.

APPENDIX 2

Sample Selection, Data and Bio-data Collection Procedures for Phase 1, 2 & 3

IDENTIFICATION AND VERIFICATION OF VENUES

Key Messages

- Community informants named 617 sites of which 459 were visited and found to be operational.
- Sex work was confirmed at 399 sites.

In Phase 1 community informants listed places where people meet new sex partners (instrument in appendix 3) while in Phase 2 visits were made to verify or reject these locations and specifically identify those locations where sex was sold and bought (instrument in appendix 4).

Phases 1 and 2 were conducted by the HIV Prevention Programme staff after four regional workshops where they were trained in the methodology. See Box for eligibility criteria for each group.

Recruitment

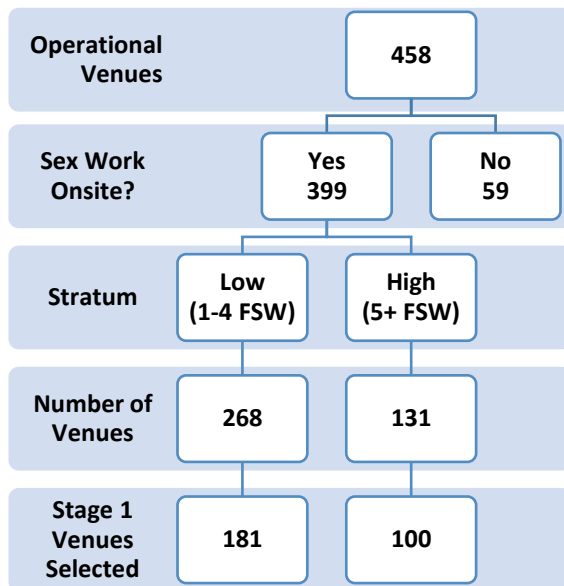
- Community informants were recruited based on targets of a wide variety of community informants (e.g. taxi drivers, security guards, street vendors). The aim was to generate a comprehensive list of venues.
- Venue informants in Phase 2 were selected onsite by interviewers based on their knowledge of the venue and included venue managers, regular patrons, venue staff and others.
- In Phase 1, over 600 venues were identified by community informants as places where people meet new sexual partners.
- In Phase 2, after deduplication and visits to the venues, 342 were found operational.
- In Phase 3, a probability sample of 1660 patrons and workers were interviewed and tested at 281 venues verified as places where people new sexual partners.

Venue Selection

- During the site verification phase of the study, a venue informant was asked to estimate the number of female workers at a venue including the number of street sex workers and exotic dancers. Survey teams comprised of the local parish Behaviours Change and Communication Programme staff attempted to visit all 671 sites named

by community informants. Of these, many were duplicate sites; others were not located, no longer in operation or located outside the parish of the team engaged in site verification. In some sites that were found and operational, a venue informant was not available or did not know how many dancers, street sex workers or other female workers were at the site.

Figure 13 : Venue selection



- At the end of the site verification phase, the study team estimated that there were 501 unique sites in operation during the period of site verification.
- Sex work was verified at 399 of the 458 operational venues during Phase 2. Based on data collected during site verification, there were 268 venues with 1-4 female sex workers (Low Stratum) and 131 venues with five or more sex workers (High Stratum). See Figure 13.

Venue Sampling Probability

Although some sites disappeared and some emerged between site verification and the bio-behavioural survey, the assumption was made, in the face of no evidence to the contrary, that the absolute number of venues did not change over the six month period and that there remained 268 “Low Stratum” venues and 131 “High Stratum” venues even if some had disappeared and been replaced with other venues. Consequently, the probability that a High Stratum venue was sampled was $100/131$, with a corresponding stage 1 sampling weight of $131/100$ or 1.31. The probability that a Low Stratum venue was sampled was $181/268$, with a corresponding sampling weight of $268/181$ or approximately 1.5.

Selection of Participants within Selected Venues

All women onsite during the interview period at selected venues were eligible for recruitment into the survey. Supervisors worked with interviewers to ensure that all women were approached and invited to participate, regardless of whether they were patrons or workers. In some cases, there were time constraints which prevented all women from being approached for an interview. A tally was kept of the number of women onsite throughout the evening so that a comparison of the number counted and the number interviewed. This information was used in the construction of sampling weights.

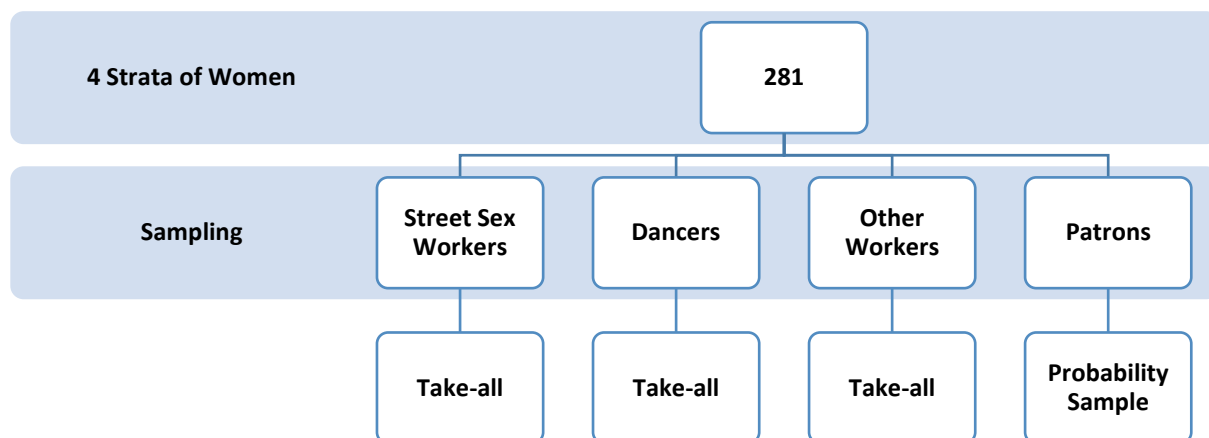
Additionally, women could be recruited to the survey by receiving a coupon inviting her to the survey. All women presenting a coupon to an interviewer were invited to participate. Street sex workers, exotic dancers and site workers were extremely willing to participate. At clubs, getting patrons to participate was sometimes a challenge. Exotic dancers were accommodated regarding their work and dancing as sometimes they had to do the survey in parts given its length. The managers of the clubs were very cooperative.

- Participants were categorized as patrons or workers based on their answer to the question: “Why did you come here today/tonight? Did you come here to work?” 1185 women answered “Yes” and were characterized as workers; the remainder (475) were characterized as patrons.

Phase 3 Sample Selection Plan

The sampling plan was to interview all women in the first three groups and a probability sample of female patrons in the expectation that most of the female sex workers would be in the first three groups but that an important minority would be among the female patrons. See Figure 14.

Figure 14: Stage 2 Selection



Although all dancers , street sex workers and other women working at a venue were eligible to participate, it was not always possible to interview everyone. Instead of assuming that everyone eligible was interviewed, the probability that a person was interviewed at a site was based on the number of women counted at each site and the number interviewed.

Four types of women were counted at a site: street sex workers, dancers, other female workers and female patrons. Each of these types were counted before each survey. The highest number counted at a given site was used as the total number of potential respondents of that type. The actual number of people recruited as that type at the site was also determined.

Probability that a participant was sampled in a selected venue

Women within selected venues were stratified into 4 strata of women: and sampled within strata. The four groups were:

- Street sex workers
- Dancers
- Other female workers and
- Female patrons.

The probability that a respondent was interviewed is the number of people of that type interviewed divided by the number of people counted. For example, if 6 patrons were counted at a site and 3 were interviewed, the probability of being sampled was $3/6=50\%$. We assume that the 3 can be considered a random sample from the 6. There were very few refusals. It is possible that the 3 who were sampled were different from the 3 who were not interviewed but the interviewers were trained to select patrons systematically based on instructions from the supervisor and not based on personal choice. We assume if not everyone of a certain type was interviewed, that those interviewed constituted a random sample of those present.

The coupon system of recruitment yielded 41 respondents, 30 of whom were workers 11 patrons. This represents 3% of the sample. Of the 41 women who were recruited via a coupon from a peer 19 were current sex workers. These were all given a probability of interview of 1.

Bio-Data Collection

Both rapid HIV and Syphilis tests were conducted at sites by trained HIV/STI Programme staff. The Standard SD Bioline 3.0 rapid method was used to test for Syphilis. Determine HIV 1/2-test kit: A finger prick test with a sensitivity of 98.22% and specificity of 99.79%

was used to test for HIV. Both rapid Syphilis and HIV testing for the survey phase adhered to the outreach testing algorithm protocols for HIV and Syphilis approved by the National Public Health Lab (NPHL). HIV and Syphilis samples were labelled with a unique identifier code generated from participants' information as well as the study generated ID. Each test was accompanied by the required pre and post-test counselling and referral where indicated.

Dried Blood spot testing was used to assess viral load among HIV positive participants. The same HIV/STI Programme staff who conducted the HIV and syphilis testing were trained in for regional workshops to conduct the plasma separation card procedure. Samples were labelled with both the study ID and HIV Programme unique identifier code.

APPENDIX 3

COMMUNITY INFORMANT INTERVIEW – ONE PER INFORMANT

C.I. #	Questions	Coding categories
	Hello. I am working on a study which is being conducted by the Ministry of Health in Jamaica to identify where better health Programmes are needed. We want to talk to persons like you in the community who we believe can assist us. The purpose of the study is to find out where people go to meet new sexual partners. Please tell us the names and locations of places where you think people meet new sexual partners. We don't want to know the names of any private residences. We are just interested in public places. If you tell us where these places are, we will visit them to see if they want to have a health Programme there. Telling us the names and locations of these public places should take between 5 and 10 minutes. We will not ask your name or any information about yourself. You will not be contacted in the future and your answers cannot be linked back to you. Your participation is completely voluntary and you may decline to answer any specific question or completely refuse to participate. We would greatly appreciate your help in responding to these questions.	
	ASK EVERYONE: Could you tell me a few places or events where people go to meet new sex partners in <this community>? USE SPACE BELOW TO LIST PLACES. THEN PROBE: • Are there places where people can pay for sex services, including oral sex, lap dances and full-body massage? • Are there streets or homes/ houses where sex workers solicit clients? • Are there other social events where people who meet each other may have a one night or brief sexual relationship? • Are there telephone contacts, websites or chat rooms available to find someone for sex? • Are there females who seek new sex partners via telephone? Do you have the number? • Are there places where men go to meet new male sex partners?	
	Record if the informant agrees or refuses to participant. 1. Agreed 2. Refused	

LIST OF SITES AND EVENTS

FOR EACH SITE OR EVENT LISTED HERE, FILL OUT A SITE/EVENT REPORT FORM. AFTER FORMS ARE COMPLETED, THANK THE RESPONDENT FOR THEIR TIME.

1. _____
2. _____
3. _____

4. _____

SITE/EVENT REPORT FORM: FOR EACH PLACE NAMED BY COMMUNITY INFORMANTS FILL IN A1 A8.

PLACE REPORT Interviewer Number: ____ Sequential CI Number: ____ Report: 1 2 3 4 5					
A1	Record the name of the site as given by community informant: _____				
A2	Circle the parish in which the interview is being conducted	Kingston 1 St Andrew 2 St Catherine 3 St. Mary 4	St Ann 5 St Elizabeth 6 Portland 7	St Thomas 8 Westmoreland 9 Hanover 10	Clarendon 11 Manchester 12 St James 13 Trelawny 14
A3	Name of community in which the site is located?	Name: _____ Census Code: _____			
A4	Select the category which best describes the place/ site	Bar 1 Night Club 2 Exotic Club /Sex Shop 3 Fast Food / Restaurant / Internet Cafe 4 Outdoor Hang Out Spot /Park /SW Site 5 Massage Parlour 6 Hotel/ Motel /Guesthouse 10 Campus Site 11 Mall/ Plaza 12 Tel #/ Internet/ Social Media Site 13 Day Of Week Event or Dance 14 Other Indoor Site 86 Other Indoor Event 87 Other Outdoor Site 88 Other Outdoor Event 89			
A5	Specify type.				
A6	Key populations at place? (Circle all that apply)	i) Female sex workers? 1 YES 2 NO ii) Trans Women? 1 YES 2 NO iii) MSM? 1 YES 2 NO			
A7	Specify in detail the address of the site/how to find it.				
A8	Record the date of Interview: Date of interview ____/____/____ Day/Month/Year				

APPENDIX 4

FORM B: SITE AND EVENT QUESTIONNAIRE

B1 PLACE ID Site NUMBER: _____

No.	Questions	Coding Categories					
B2- B4	B2 Interviewer Number: _____ (DD/MM/YY): ____/____/____	B3 Initials: _____ B4 Date					
B5	<table border="1"> <tr> <td>Parish Of Place Circle 1</td> <td> KINGSTON 1 ST ANDREW 2 ST CATHERINE 3 </td> <td> ST. MARY 4 ST ANN 5 ST ELIZABETH 6 PORTLAND 7 </td> <td> ST THOMAS 8 WESTMORELAND 9 HANOVER 10 </td> <td> CLARENDON 11 MANCHESTER 12 ST JAMES 13 TRELAWNY 14 </td> </tr> </table>	Parish Of Place Circle 1	KINGSTON 1 ST ANDREW 2 ST CATHERINE 3	ST. MARY 4 ST ANN 5 ST ELIZABETH 6 PORTLAND 7	ST THOMAS 8 WESTMORELAND 9 HANOVER 10	CLARENDON 11 MANCHESTER 12 ST JAMES 13 TRELAWNY 14	
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B6a	Name and Type of PLACE from CI Reports:						
B6b	Actual Name of Place:						
B7	Actual Community Where Site/Event Is Located:						
B8	Correct Address / Directions						
B9	<table border="1"> <tr> <td>Type Of Place</td> <td> Bar 1 Night club 2 Exotic club / sex shop 3 Fast food / restaurant / internet cafe 4 Outdoor hang out spot / park/ sw site 5 Massage parlour 6 Hotel/ motel /guesthouse 10 </td> <td> Campus site 11 Mall/ plaza 12 Tel/internet/ social media site 13 Day of week event or dance 14 Other indoor site 86 Other indoor event 87 Other outdoor site 88 Other outdoor event 89 </td> </tr> </table>	Type Of Place	Bar 1 Night club 2 Exotic club / sex shop 3 Fast food / restaurant / internet cafe 4 Outdoor hang out spot / park/ sw site 5 Massage parlour 6 Hotel/ motel /guesthouse 10	Campus site 11 Mall/ plaza 12 Tel/internet/ social media site 13 Day of week event or dance 14 Other indoor site 86 Other indoor event 87 Other outdoor site 88 Other outdoor event 89			
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B10	Describe Type Of Site/Event In Words.						

No.	Questions	Coding Categories
B11	Role Of Respondent at Site	STAFF 1 SECURITY GUARD 2 PATRON 3 VENDOR IN AREA 4 SEX WORKER 5 STUDENT 6 B11A OTHER (SPECIFY) : _____ 7
B12	Interviewer Opinion: How knowledgeable is this respondent likely to be about the activities and patrons at the site?	EXTREMELY KNOWLEDGEABLE 1 KNOWLEDGEABLE 2 NOT VERY KNOWLEDGEABLE 3

Hello. I am working on a study approved by the Ministry of Health in Jamaica. We want to talk to people like you who know about this community. The purpose of the study is to identify where better health Programmes are needed to prevent the spread of diseases that are transmitted by sex. I would like to ask you some questions about activities that occur at this place, the people who come here, and Programmes that may take place here. The interview should take between 30-40 minutes of your time. I won't ask your name or any other identifying information. Your participation is completely voluntary and you may decline to answer any question or completely refuse to participate. We appreciate your help responding to these questions, even though we cannot financially compensate you for your time. You may not personally benefit directly from the study, but the results may be used to plan a new health Programme for this area. This is what we will do with the information you give us. Your answers will be recorded on a paper that only identifies this place with a number. Your name will not be recorded anywhere and we won't ask any personal information about you except to make sure you are age 18 or older. We are asking people these questions at more than one hundred places in this area. The questionnaires will be kept in a locked cabinet. The only people who will see the questionnaires are people working on this study. An ethical review board has reviewed this study. If you have any questions please feel free to contact Mrs. Rahannah Khan at the Ministry of Health 876-

B13	Questions To Respondent Begin Here: ARE YOU WILLING TO ANSWER THESE QUESTIONS? If Respondent Refuses, Find Another.		YES 1 NO 2
B14a B14b	Was An Interview Completed? If Not Why Not?	YES 1 SITE OPERATIONAL BUT NO WILLING RESPONDENT 2 SITE OPERATIONAL BUT ALL POTENTIAL RESPONDENTS TOO YOUNG 3 SITE CLOSED TEMPORARILY 4 SITE CLOSED PERMANENTLY OR NO LONGER A SITE 5 OTHER(SPECIFY REASON): _____ 8	

No.	Questions	Coding Categories																																							
B15	HOW MANY YEARS HAS THIS SITE BEEN IN OPERATION?	< 1 YEAR 1 1-2 YEARS 2 3-5 YEARS 3 MORE THAN 5 YEARS 4 Not applicable 9																																							
B16a - B16h	WHICH TYPES OF ACTIVITIES TAKE PLACE HERE? Circle Yes or No For Each.	<table><tr><th></th><th>YES</th><th>NO</th></tr><tr><td>A. ALCOHOL (BEER OR TONIC WINE) CONSUMED</td><td>1</td><td>2</td></tr><tr><td>B. ECSTASY / MOLLY USED ONSITE</td><td>1</td><td>2</td></tr><tr><td>C. CRACK/COCAINE USED ONSITE</td><td>1</td><td>2</td></tr><tr><td>D. EXOTIC OR GO-GO DANCING</td><td>1</td><td>2</td></tr><tr><td>E. SEX ON STAGE</td><td>1</td><td>2</td></tr><tr><td>F. MUSIC (LIVE OR RECORDED)</td><td>1</td><td>2</td></tr><tr><td>G. MASSAGE</td><td>1</td><td>2</td></tr><tr><td>H. PEOPLE WAITING FOR A BUS OR TAXI</td><td>1</td><td>2</td></tr><tr><td>2OTHER</td><td></td><td></td></tr></table>		YES	NO	A. ALCOHOL (BEER OR TONIC WINE) CONSUMED	1	2	B. ECSTASY / MOLLY USED ONSITE	1	2	C. CRACK/COCAINE USED ONSITE	1	2	D. EXOTIC OR GO-GO DANCING	1	2	E. SEX ON STAGE	1	2	F. MUSIC (LIVE OR RECORDED)	1	2	G. MASSAGE	1	2	H. PEOPLE WAITING FOR A BUS OR TAXI	1	2	2OTHER											
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B17a - B1b	I HAVE BEEN TOLD THAT PEOPLE MEET SEXUAL PARTNERS AT PLACES LIKE THIS. DO..... Read List.	<table><tr><th>YES</th><th>NO</th><th>DK</th></tr><tr><td colspan="3">A. DO MEN MEET NEW FEMALE SEXUAL PARTNERS HERE?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr><tr><td colspan="3">B. DO WOMEN MEET NEW MALE SEX PARTNERS HERE?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr><tr><td colspan="3">C. DO MEN COME HERE TO BUY SEX FROM WOMEN?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr><tr><td colspan="3">D. DO TRANS WOMEN MEET NEW MALE SEX PARTNERS HERE?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr><tr><td colspan="3">E. DOES SOMEONE ONSITE HELP PARTNERS HOOK UP?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr><tr><td colspan="3">F. DO FEMALE SEX WORKERS COME HERE TO FIND CUSTOMERS?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr></table>	YES	NO	DK	A. DO MEN MEET NEW FEMALE SEXUAL PARTNERS HERE?			1	2	8	B. DO WOMEN MEET NEW MALE SEX PARTNERS HERE?			1	2	8	C. DO MEN COME HERE TO BUY SEX FROM WOMEN?			1	2	8	D. DO TRANS WOMEN MEET NEW MALE SEX PARTNERS HERE?			1	2	8	E. DOES SOMEONE ONSITE HELP PARTNERS HOOK UP?			1	2	8	F. DO FEMALE SEX WORKERS COME HERE TO FIND CUSTOMERS?			1	2	8
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B17c - B17j	Continued.... I HAVE BEEN TOLD THAT PEOPLE MEET SEXUAL PARTNERS AT PLACES LIKE THIS. DO..... Read List.	<table><tr><th colspan="3">YES NO DK</th></tr><tr><td colspan="3">G. DO ANY MALE STAFF MEET NEW SEXUAL PARTNERS HERE?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr><tr><td colspan="3">H. DO ANY FEMALE STAFF MEET NEW SEXUAL PARTNERS HERE?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr><tr><td colspan="3">I. CAN PEOPLE HAVE SEX ONSITE?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr><tr><td colspan="3">J. DO GAY MEN MEET MALE SEX PARTNERS HERE?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr><tr><td colspan="3">K DO GAY MEN WHO ARE SEX WORKERS COME HERE TO FIND CUSTOMERS?</td></tr><tr><td>1</td><td>2</td><td>8</td></tr></table>	YES NO DK			G. DO ANY MALE STAFF MEET NEW SEXUAL PARTNERS HERE?			1	2	8	H. DO ANY FEMALE STAFF MEET NEW SEXUAL PARTNERS HERE?			1	2	8	I. CAN PEOPLE HAVE SEX ONSITE?			1	2	8	J. DO GAY MEN MEET MALE SEX PARTNERS HERE?			1	2	8	K DO GAY MEN WHO ARE SEX WORKERS COME HERE TO FIND CUSTOMERS?			1	2	8						
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No.	Questions	Coding Categories																														
B18	WHEN DO FEMALE SEX WORKERS COME HERE?	EVERY DAY /NIGHT 1 NONE/ NOT AT ALL 2 SOME DAYS/NIGHTS 3																														
B19	DOES THIS PLACE HAVE SPECIAL DAYS OR NIGHTS?	NO SPECIAL DAYS OR NIGHTS: 0 YES, SPECIAL TIMES 1 Specify in next section																														
B20	A NAME OF SPECIAL: _____ B HOW OFTEN: _____ TIME: ____AM/PM TO ____AM/PM																															
B21	A NAME OF SPECIAL: _____ B HOW OFTEN: _____ TIME: ____AM/PM TO ____AM/PM																															
B22a - B22e	A WHAT IS THE BUSIEST DAY OF THE WEEK HERE? Circle 1. B_C WHAT ARE THE BUSIEST HOURS ON THAT DAY? D_E DURING THAT TIME, HOW MANY MEN AND WOMEN DO YOU EXPECT TO BE SOCIALIZING AT THIS SITE? Read Options.	A : MONDAY TUESDAY WEDNESDAY THURSDAY FRIDAY SATURDAY SUNDAY B: BUSY HOURS BEGIN: ____ AM/ PM C HOURS END: ____AM /PM D MEN: <table border="0"> <tr><td><25</td><td>1</td><td>1</td></tr> <tr><td>25-50</td><td>2</td><td>2</td></tr> <tr><td>51-100</td><td>3</td><td>3</td></tr> <tr><td>101-150</td><td>4</td><td>4</td></tr> <tr><td>151-200</td><td>5</td><td>5</td></tr> <tr><td>> 200</td><td>6</td><td>6</td></tr> </table> E WOMEN :	<25	1	1	25-50	2	2	51-100	3	3	101-150	4	4	151-200	5	5	> 200	6	6												
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B23A -B23I	WHAT TYPES OF WORKERS ARE AT THIS SITE/EVENT? If There Are Zero Workers of a Certain Type, Write Zero "0".	<table border="0"> <tr><td></td><td>YES</td><td>NO</td></tr> <tr><td>A. ANY FEMALE DANCERS?</td><td>1</td><td>2</td></tr> <tr><td>B. IF YES: NUMBER OF FEMALE DANCERS ON A BUSY NIGHT: _____</td><td></td><td></td></tr> <tr><td>C. ANY MALE DANCERS?</td><td>1</td><td>2</td></tr> <tr><td>D. IF YES: NUMBER OF MALE DANCERS ON A BUSY NIGHT: _____</td><td></td><td></td></tr> <tr><td>E. ANY PAID STAFF EXCLUDING DANCERS?</td><td>1</td><td>2</td></tr> <tr><td>F. IF YES NUMBER OF MALE STAFF EXCLUDING DANCERS: _____</td><td></td><td></td></tr> <tr><td>G. IF YES NUMBER OF FEMALE STAFF EXCLUDING DANCERS: _____</td><td></td><td></td></tr> <tr><td>H. ANY PARTY GIRLS?</td><td>1</td><td>2</td></tr> <tr><td>I. IF YES NUMBER OF PARTY GIRLS ENTERTAINING CUSTOMERS AT THE EVENT _____</td><td></td><td></td></tr> </table>		YES	NO	A. ANY FEMALE DANCERS?	1	2	B. IF YES: NUMBER OF FEMALE DANCERS ON A BUSY NIGHT: _____			C. ANY MALE DANCERS?	1	2	D. IF YES: NUMBER OF MALE DANCERS ON A BUSY NIGHT: _____			E. ANY PAID STAFF EXCLUDING DANCERS?	1	2	F. IF YES NUMBER OF MALE STAFF EXCLUDING DANCERS: _____			G. IF YES NUMBER OF FEMALE STAFF EXCLUDING DANCERS: _____			H. ANY PARTY GIRLS?	1	2	I. IF YES NUMBER OF PARTY GIRLS ENTERTAINING CUSTOMERS AT THE EVENT _____		
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No.	Questions	Coding Categories																																																				
B23J B23Q	Now I would like to ask you about how many female sex workers are here at a busy time. This could include street sex workers, dancers, other workers, or female patrons who socialize here. Write zero (0) where there are zero sex workers of a certain type																																																					
B24A- B24L	In the past 12 months (since put in date), have there been any hiv prevention activities at this site? Has there been.....Ask Each.	<table border="0"> <thead> <tr> <th></th><th>YES</th><th>NO</th><th>DK</th></tr> </thead> <tbody> <tr> <td>A. ANY HIV/AIDS PREVENTION?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>B. ANY CONDOM PROMOTION?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>C ANY PERSONS ACTUALLY TESTED ONSITE FOR HIV?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>D ANY SAFER SEX EDUCATION BY OUTREACH WORKERS?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>E. ANY OUTREACH THROUGH SEX WORK PEER EDUCATORS?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>F. ANY OUTREACH THROUGH INFLUENTIAL STAFF?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>G. ANY OUTREACH OR PREVENTION BY MOHW?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>H. ANY OUTREACH OR PREVENTION BY JASL?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>I. ANY OUTREACH OR PREVENTION BY CHILDREN FIRST?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>J. ANY OUTREACH BY ANY OTHER ORGANIZATION?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>K. IF YES: WHAT ORGANIZATION: _____</td><td></td><td></td><td></td></tr> <tr> <td>L. ANY OTHER SURVEYS ABOUT HIV PREVENTION AT SITE?</td><td>1</td><td>2</td><td>9</td></tr> </tbody> </table>		YES	NO	DK	A. ANY HIV/AIDS PREVENTION?	1	2	9	B. ANY CONDOM PROMOTION?	1	2	9	C ANY PERSONS ACTUALLY TESTED ONSITE FOR HIV?	1	2	9	D ANY SAFER SEX EDUCATION BY OUTREACH WORKERS?	1	2	9	E. ANY OUTREACH THROUGH SEX WORK PEER EDUCATORS?	1	2	9	F. ANY OUTREACH THROUGH INFLUENTIAL STAFF?	1	2	9	G. ANY OUTREACH OR PREVENTION BY MOHW?	1	2	9	H. ANY OUTREACH OR PREVENTION BY JASL?	1	2	9	I. ANY OUTREACH OR PREVENTION BY CHILDREN FIRST?	1	2	9	J. ANY OUTREACH BY ANY OTHER ORGANIZATION?	1	2	9	K. IF YES: WHAT ORGANIZATION: _____				L. ANY OTHER SURVEYS ABOUT HIV PREVENTION AT SITE?	1	2	9
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B25	In the past 12 months, how often have condoms been available here?	ALWAYS 1 SOMETIMES 2 NEVER 3 DON'T KNOW 8																																																				
B26	Can i get a condom here now? If yes, can i see one?	YES, CONDOMS HERE BUT YOU CAN'T SEE ONE 1 YES, CONDOMS HERE AND A CONDOM WAS SEEN 2 NO CONDOMS HERE NOW 3																																																				
B27a B27b B27c B27d	Interviewer Observation: Evidence Of HIV/AIDS Prevention Activities Noted By Interviewer at the Site	<table border="0"> <thead> <tr> <th></th><th>YES</th><th>NO</th></tr> </thead> <tbody> <tr> <td>A. ANY HIV/AIDS POSTERS DISPLAYED?</td><td>1</td><td>2</td></tr> <tr> <td>B ANY CONDOM PROMOTION OR USE POSTERS?</td><td>1</td><td>2</td></tr> <tr> <td>C. CONDOMS VISIBLE?</td><td>1</td><td>2</td></tr> <tr> <td>D. CONDOMS FOR SALE ON SITE?</td><td>1</td><td>2</td></tr> </tbody> </table>		YES	NO	A. ANY HIV/AIDS POSTERS DISPLAYED?	1	2	B ANY CONDOM PROMOTION OR USE POSTERS?	1	2	C. CONDOMS VISIBLE?	1	2	D. CONDOMS FOR SALE ON SITE?	1	2																																					
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B28	Would you like to give contact information in case we want to do some outreach activities here?	DECLINED TO GIVE CONTACT INFORMATION ____ NAME: _____ MOBILE: _____																																																				

No.	Questions	Coding Categories
B29	Interviewer Opinion How Supportive Are Gatekeepers Regarding Safe Sex Practices at The Site?	VERY SUPPORTIVE 1 SUPPORTIVE 2 NOT SUPPORTIVE 3 VERY UNSUPPORTIVE 4 DO NOT KNOW 5 NO GATEKEEPERS 9
B30	Interviewer's Prevention Suggestions for Place:	

APPENDIX 5

MOH PLACE BIO-BEHAVIORAL SURVEILLANCE SURVEY 2023

QUESTIONNAIRE # _____ PLACE SITE ID #: _____ PARTICIPANT CODE: _____

No.	INTERVIEW DETAILS				
1. 2.	Parish of Site or Event: CirCle one	SOUTHEAST KINGSTON 1 ST ANDREW 2 ST CATHERINE 3 ST THOMAS 4	NORTHEAST ST. MARY 5 ST ANN 6 PORTLAND 7	WESTERN WESTMORELAND 8 HANOVER 9 ST JAMES 10 TRELAWNY 11	SOUTHERN ST ELIZABETH 12 CLARENDON 13 MANCHESTER 14
3.	Record the name of the community in which the site or event is located		Name of Community: _____ Census code: _____		
4.	Original Site Name		Name of Site: _____		
5.	Name of replacement site, if necessary		Name of Site: _____		
6.	TYPE OF SITE	BAR 1 NIGHT CLUB 2 EXOTIC CLUB / SEX SHOP 3 FAST FOOD / RESTAURANT / INTERNET CAFE 4 OUTDOOR HANG OUT SPOT / PARK/ SW SITE 5 HOTEL/ MOTEL /GUESTHOUSE 10 CAMPUS SITE 11 MALL/ PLAZA 12 INTERNET/ SOCIAL MEDIA SITE 13 DAY OF WEEK EVENT OR DANCE 14			I5b. SPECIFY 90: _____

		OTHER INDOOR SITE 86 OTHER INDOOR EVENT 87 OTHER OUTDOOR SITE 88 OTHER OUTDOOR EVENT 89 OTHER (SPECIFY) 90 (go to I5b)	
7.	INTERVIEWER	INTERVIEWER INITIALS: _____	
8.	DATE TODAY	____/____/_____ day month year	
9.	TIME INTERVIEW BEGINS	Hour: ____ Min: ____ Circle: AM or PM	
10.	PERSON APPROACHED AS: CIRCLE ONE ANSWER. INDICATE THE TYPE OF RESPONDENT THEN COUNT THE NUMBER OF THAT TYPE ONSITE AT THIS TIME AND ENTER THE NUMBER:	STREET SEX WORKER 1 NUMBER COUNTED AT SITE ____ VENUE/EVENT BASED SEX WORKER (party, massage parlours, etc.) 2 NUMBER COUNTED AT SITE ____ SEX WORKER DANCER 3 NUMBER COUNTED AT SITE ____ VENUE WORKER (bartender, waitress, etc) 4 NUMBER COUNTED AT SITE ____ PATRON (women at site with SWs) 5 NUMBER COUNTED AT SITE ____ HOME /COMMUNITY BASED SEX WORKER 6 NUMBER COUNTED AT SITE ____	FSW RECRUITED BY FOR INCENTIVE 7 COUPON NUMBER____ A MAN AT MSM EVENTS CREATED BY MSM PEER LINKS 8 NUMBER COUNTED AT SITE ____ TRANSGENDER WOMAN 9 NUMBER COUNTED AT SITE ____ MSM RECRUITED BY COUPON 10 COUPON NUMBER____ TG BASED ON OBSERVATION: 11 NUMBER COUNTED AT SITE ____
INTRODUCTION OF STUDY			
Hello, I am working on a study which is being conducted by the Jamaica Ministry of Health & Wellness to identify where better health Programmes are needed. The study seeks to get information on sexual practices and sexually transmitted infections. I would like to offer you the opportunity to participate in the survey.			

ELIGIBILITY DETERMINATION				
E1	Sex of respondent (as identified by respondent) IF MALE, END INTERVIEW	MALE 1 FEMALE 2 TRANS WOMAN (Male to Female) 3		
E2	ASK EVERYONE: What is your age? What is your date of birth? IF < 16, END INTERVIEW	A. AGE IN YEARS: ____/ B. BIRTHDATE. / ____ / ____ / ____ / DAY MONTH YEAR		
E3	IF AGE 16-17, Are you here with a parent or on a family errand?	END, NOT ELIGIBLE IF : YES 1 CONTINUE IF: NO 2		
E4	ASK EVERYONE: Have you ever participated in a survey conducted by the Ministry of Health & Wellness?	CONTINUE IF: YES 1 REQUEST INFORMED CONSENT IF: NO 2		
E5	IF YES, did you participate in a Ministry of Health & Wellness survey carried out between Dec. 2023 & Jan. 2024 in which persons got tested for HIV and Syphilis?	END INTERVIEW IF: YES 1 REQUEST INFORMED CONSENT IF: NO 2		
E6	Were you invited to participate in this survey by receiving a green or yellow coupon? (Ask to see the coupon)	No 1 Yes GREEN 2 YES YELLOW 3		
E7	If yes. Who invited/ gave you the coupon?	A manager, supervisor or other person from a club, bar or other venue (GREEN) 1 A female friend or acquaintance (YELLOW) 2 Other (state) _____ 3		
No.	REQUEST INFORMED CONSENT. RESPONDENT MUST INITIAL CONSENT FORM			
E7	Just to confirm, I need to know if you are willing to.... IF 'NO' TO ANY EXCEPT D, END INTERVIEW.		YES	NO
		A. ANSWER SURVEY QUESTIONS?	1	2
		B. GET AN HIV RAPID TEST?	1	2
		C. GET A SYPHILIS RAPID TEST?	1	2
E8	IF NOT WILLING: Why not? END INTERVIEW IF NOT WILLING.	D. GET A VIRAL LOAD TEST		
		REASON: _____ _____		
DEMOGRAPHIC INFORMATION				

D1	Parish of residence : Circle one answer	SOUTHEAST KINGSTON 1 ST ANDREW 2 ST CATHERINE 3 ST THOMAS 4	NORTH EAST ST. MARY 5 ST ANN 6 PORTLAND 7	WESTERN WESTMORELAND 8 HANOVER 9 ST JAMES 10 TRELAWNY 11	SOUTHERN ST ELIZABETH 12 CLARENDON 13 MANCHESTER 14 OUTSIDE OF JAMAICA 15 Please note which country _____
D2	What is the highest grade you completed in primary school, in secondary school, and at post-secondary tertiary level? FILL IN NUMBER OF YEARS FOR EACH (B-G).		A. NONE B. PRIMARY _____ C. JUNIOR HIGH /ALL AGE _____ D. HIGH SCHOOL/ SECONDARY _____ E. TERTIARY (College or University) _____ F. VOCACTIONAL/SKILLS TRAINING _____ G. NO RESPONSE		
D3	Are you in school now?		YES 1 NO 2		
No.	REASON FOR BEING AT SITE				
R1	Why did you come here today/tonight? Did you come to ... CIRCLE YES OR NO FOR EACH.		YES	NO	
		A. TO SOCIALIZE	1	2	
		B. TO DRINK ALCOHOL?	1	2	
		C. TO WORK?	1	2	
		D. LOOK FOR A SEXUAL PARTNER?	1	2	
		E. RECRUITED TO PARTICIPATE IN STUDY	1	2	
R2	How often do you come to this site? READ OPTIONS CIRCLE ONE ANSWER.	EVERYDAY 1 4-6 TIMES PER WEEK 2 2-3 TIMES PER WEEK 3 1 TIME PER WEEK 4	2-3 TIMES PER MONTH 5 1 TIME PER MONTH 6 LESS THAN ONCE A MONTH 7 THIS IS MY FIRST VISIT 8		
R3	How long have you been visiting physical sites to look for sexual partners?	NEVER 0 LESS THAN A WEEK 1 LESS THAN A MONTH 2 2 TO 6 MONTHS 3	6 TO 12 MONTHS 4 1-5 YEARS 5 OVER 5 YEARS 6 OVER 10 YEARS 7		

R4	How many other public places have you been to today to socialize, drink alcohol or look for a person to have sex with? How many other public places do you plan to go to today or tonight to socialize, drink alcohol or look for a person to have sex with? PUT "0" IF NO OTHER PLACES.	A. OTHER PLACES BEEN TO: _____ B. OTHER PLACES WILL GO TO: _____ -
----	---	--

R5	How often do you look for male sex partners through the telephone, a website or on social media?	EVERYDAY 1 4-6 TIMES PER WEEK 2 2-3 TIMES PER WEEK 3 1 TIME PER WEEK 4	2-3 TIMES PER MONTH 5 1 TIME PER MONTH 6 LESS THAN ONCE A MONTH 7 NEVER 8
THE NEXT FEW QUESTIONS ARE ABOUT SEXUAL PRACTICES			
S1	Have you ever had sex? IF NO, SKIP TO Sy1 (Sex means intercourse in the anus mouth or vagina)	YES 1 NO 2	
S2	How old were you the first time you had sex?	AGE FIRST HAD SEX : _____	
S3	Do you have a main <u>male</u> sexual partner now? Do you have a main <u>female</u> partner now? (Main partner is a live in or visiting relationship)	YES 1 NO 2 A. MAIN MALE PARTNER 1 2 B. MAIN FEMALE PARTNER 1 2	
S4	Are you currently married or living with a partner? IF NO: Have you ever been married or lived with a sexual partner?	CURRENTLY MARRIED / LIVING WITH 1 PREVIOUSLY MARRIED/LIVED WITH, NOT NOW 2 NEVER MARRIED OR LIVED WITH 3	
S5	The last time you had sex with your (most recent) main male partner; did you and your partner use a condom? (Main partner is a live in partner or a visiting relationship) IF YES SKIP TO S7	YES NO YES 1 NO 2 NO MAIN PARTNER 9	

S6	What were the main reasons for you not using condoms all the time with your main partner? (Probe: any other reason?)	Love or trust partner 1 Don't like using condoms 2 Did not have one/ Could not get one 3 Partner does not like using condoms 4 Did not feel to use one/could not bother 5 Allergic to condoms/allergic to some brands 6 Trying to have a child 7 Using the pill /other contraceptive method 8 Have only one partner 9 Other Reason(s) 10 Specify other _____ _____
S6b		
S7	Do you believe that your main partner (any of your main partners) has had sex with another person in the past 6 months?	YES 1 NO 2 NO MAIN PARTNER 9
S8	In the past <u>12 months</u> , have you had sex with a person who you never had sex with before?	YES 1 NO 2
S9	In the past 6 months have you had sex with only men, only women, or both men and women?	ONLY MEN 1 ONLY WOMEN 2 BOTH MEN & WOMEN 3 HAVE NOT HAD SEX IN PAST SIX MONTHS 4
S10	Now I am going to ask you a few more questions about the number of men you have had sex with. In total, how many men did you have sex with in the past 7 days? This includes all the persons you have had sex with even if just one time.	TOTAL IN PAST 7 DAYS: _____
S11	In total, how many different men did you have sex with in the past 4 weeks? This includes all the persons you have had sex with since (State the date one month before day of interview) . THIS NUMBER SHOULD BE THE SAME OR GREATER AS S10. IF NOT, PROBE!	TOTAL MEN PAST 4 WEEKS: _____
S12	Of the _____ (INSERT FROM S11) men you had sex with in the past 4 weeks, how many of these were new persons you never had sex with before? That is, you had sex with them for the first time in the past 4 weeks.	NEW IN PAST 4 WEEKS: _____

	THIS NUMBER SHOULD BE THE SAME OR <u>LESS</u> AS IN S10. IF NOT, PROBE!	
S13 A	In total, how many persons (men and women) did you have sex with in the past 12 months? This includes all the persons you have had sex with. HELP PERSON ESTIMATE.	TOTAL MEN IN PAST 12 MONTHS_____ TOTAL WOMEN IN PAST 12 MONTHS_____
S13 B	IF RESPONDENT HAD SEX WITH MORE THAN ONE WOMAN IN THE PAST 12 MONTHS ASK: How many women did you have sex with in the past 4 weeks?	TOTAL WOMEN PAST 4 WEEKS:_____
S14	Do you have a (male) condom with you now? IF YES: May I see it?	YES, condom seen 1 YES, not seen 2 NO 3
S15	Would you say it is easy, very easy, difficult or very difficult to get a condom?	EASY 1 VERY EASY 2 DIFFICULT 3 VERY DIFFICULT 4
THE NEXT SET OF QUESTIONS ARE ON SEXUAL AND REPRODUCTIVE HEALTH AND FAMILY PLANNING		
F1	WOMEN ONLY: How many times have you been pregnant?	_____
F2	WOMEN ONLY: A Pap Smear is a test for cancer of the cervix which is done during a pelvic examination by a doctor or nurse. How long has it been since your last Pap smear, if ever?	Never had one 1 Less than 1 year 2 More than 1 year, less than 2 years ago 3 2 to 3 years 4 More than 3 years ago 5 Does not know / not sure 6
F2a	MEN ONLY: Are you circumcised?	YES 1 NO 2
F3	MEN AND WOMEN: In your last 10 sexual experiences,	All 10 times 1 8-9 times 2 7-6 times 3

	how many times was a condom used? READ OPTIONS	5-3 times 4 2-1 times 5 Never used a condom at all 6 Not applicable 9																				
F4	Are you and your partner <u>currently</u> (last 30 days) using a method of contraception/birth control or doing anything to prevent pregnancy? If 'No' got to C1	YES 1 NO 2 No Partner 9																				
F5	If yes, which method(s) are you currently using? Circle all that apply	Female sterilization, tubal ligation (<i>tie-off</i>) 1 Implant (Jadelle) 2 IUD/ Coil 3 Injection (Depo) 4 Pill 5 Condom 6 Emergency Contraceptive Protection (a.k.a. the Morning After Pill) 7 Spermicides 8 Withdrawal 9 Rhythm, Calendar, Billings (the safe period) 10 Other(F6b) 11 Specify other: _____																				
COMMERCIAL SEX AND CONDOM USE																						
C1	ASK EVERYONE WHO HAS EVER HAD SEX. Sometimes people have sex with someone in exchange for gifts or help with expenses or cash money. Have you....	<table border="1"> <thead> <tr> <th></th><th></th><th>YES</th><th>NO</th></tr> </thead> <tbody> <tr> <td>A</td><td>Ever received help with expenses?</td><td>1</td><td>2</td></tr> <tr> <td>B</td><td>Ever <u>received</u> money in exchange for sex?</td><td>1</td><td>2</td></tr> <tr> <td>C</td><td>Ever paid money for sex?</td><td>1</td><td>2</td></tr> <tr> <td>D</td><td>Ever received gifts or favors in exchange for sex?</td><td>1</td><td>2</td></tr> </tbody> </table>			YES	NO	A	Ever received help with expenses?	1	2	B	Ever <u>received</u> money in exchange for sex?	1	2	C	Ever paid money for sex?	1	2	D	Ever received gifts or favors in exchange for sex?	1	2
		YES	NO																			
A	Ever received help with expenses?	1	2																			
B	Ever <u>received</u> money in exchange for sex?	1	2																			
C	Ever paid money for sex?	1	2																			
D	Ever received gifts or favors in exchange for sex?	1	2																			
C2	If <u>RECEIVED</u> CASH MONEY: Have you RECEIVED cash money from a person for sex ...?	<table> <tr> <td></td><td>YES</td><td>NO</td><td>NA</td></tr> <tr> <td>A. IN THE PAST 3 MONTHS?</td><td>1</td><td>2</td><td>9</td></tr> <tr> <td>B. IN THE PAST 6 MONTHS?</td><td>1</td><td>2</td><td>9</td></tr> </table>		YES	NO	NA	A. IN THE PAST 3 MONTHS?	1	2	9	B. IN THE PAST 6 MONTHS?	1	2	9								
	YES	NO	NA																			
A. IN THE PAST 3 MONTHS?	1	2	9																			
B. IN THE PAST 6 MONTHS?	1	2	9																			

	If No to both skip to C8					
C3	In which parishes did you receive cash for sex in the <u>past 6 months</u> ? CirCLE all that Apply	SOUTHEAST KINGSTON 1 ST ANDREW 2 ST CATHERINE 3 ST THOMAS 4	NORTHEAST ST. MARY 5 ST ANN 6 PORTLAND 7	WESTERN WESTMORELAND 8 HANOVER 9 ST JAMES 10 TRELAWNY 11	SOUTHERN ST ELIZABETH 12 CLARENDON 13 MANCHESTER 14 OUTSIDE OF JAMAICA 15 Which countries?	
C4	In the past <u>3 months</u> , at how many different places did you look for men to pay you money for sex?	NUMBER OF SITES: _____				
C5	How long have you been involved in Sex work?	LESS THAN A WEEK 1 LESS THAN A MONTH 2 2 TO 6 MONTHS 3 6 TO 12 MONTHS 4			1-5 YEARS 5 OVER 5 YEARS 6 OVER 10 YEARS 7	
C6	What do you think are the main 3 reasons for you getting involved in sex work? Read aloud and circle Three Only	GOT PREGNANT AND DID NOT FINISH SCHOOL 1 COULDN'T FIND A JOB 2 POVERTY 3 CHILDREN TO CARE FOR 4 NO FINANCIAL SUPPORT FROM BABY FATHER 5 NO FAMILY SUPPORT 6			GREW UP WITHOUT PARENTS 7 GREW UP /SPENT TIME IN CHILDREN'S HOME 8 WAS SEXUALLY ABUSED AS A CHILD 9 HAVE A DRUG OR ALCOHOL PROBLEM 10 WANTED TO BUY A CELL PHONE OR OTHER EXPENSIVE ITEM 11 RAN AWAY FROM HOME 12	
C7	Is Sex work your main source of income?			YES 1 NO 2		

C8	How old were you the first time you had sex with a paying client?	Age: _____
C9	The last time you had sex with a new paying client, did you and your client/customer use a condom?	YES 1 NO 2
C10	The last time you had sex with a regular paying client, did you and your client/customer use a condom?	YES 1 NO 2 NO REGULAR PAYING CLIENT 9
CURRENT SYMPTOMS AND PARTICIPATION IN HEALTH PORGRAMS AND TESTING: ASK EVERYONE		
Sy1	A. Have you had a genital discharge, ulcer, sore, warts, pain or burning when you urinate anytime <u>in the past 3 months</u>? B. IF NO: Anytime <u>in the past 12 months</u>? IF NO: CIRCLE 3's (N/A) IN Sy2.	
		YES NO NA A. Past 3 months? 1 2 3 B. Past 12 months? 1 2 3
Sy2	Which of the following did you do the last time you had a discharge, ulcer or sore? READ EACH.	YES NO N/A
		A. GET DRUGS WITHOUT PRESCRIPTION? 1 2 3
		B. GO TO A PUBLIC HEALTH CARE PROVIDER? 1 2 3
		C. GO TO A PRIVATE DOCTOR? 1 2 3
		D. TREAT WITH A HOME REMEDY 1 2 3
		E. RESPONDED/TREATED BASED ON INTERNET/ GOOGLE SEARCH 1 2 3
Sy3	Have you been told by a health care worker in the past 12 months (GIVE MONTH YEAR) that you had a sexually transmitted infection? IF YES, CIRCLE one answer. B. If NO, Have you <u>ever</u> been told? Circle 2 if ever or 3 if never.	YES, PAST 12 MONTHS 1 YES, BUT NOT IN PAST 12 MONTHS 2 NEVER 3
Sy4	A. Do you know where you can go if you wish to receive an HIV test?	YES 1 NO 2
	B. Have you ever been tested for HIV? If never tested skip to SY8	YES 1 NO 2 Never had a test 3

Sy5	Not including today, have you had an HIV test in the past 12 months? CIRCLE one	YES, IN PAST 12 MONTHS 1 YES, EVER HAD A TEST 2
Sy6	The last time you were tested, did you get your result? If yes , were you counselled when you got your result?	DID NOT GET RESULT 1 GOT RESULT AND WAS <u>COUNSELED</u> 2 GOT RESULT BUT <u>NOT COUNSELED</u> 3 NEVER TESTED 9
Sy7	ASK BOTH OF THESE QUESTIONS: Did you tell your main partner the results of your last HIV test (not including the test today)? CIRCLE ONE ANSWER. Did you tell any of your friends, family or co-workers the results of your last HIV test (not including the test today)?	A. MAIN PARTNER? YES 1 NO 2 NA 9 B. ANY FRIENDS, FAMILY? YES 1 NO 2 NA 9
Sy8a Syb	IF NO HIV TEST What is the main reason why you have never gotten tested for HIV?	Don't Think I am at Risk 1 Never thought about it 2 Fear of Positive Result 3 No money or time to get tested 4 Fear that my results will not be kept confidential by health care workers 5 Fear of Health Care Workers treating me badly 6 Didn't know I could get tested for HIV 7 Other Specify _____ 9

A FEW QUESTIONS ON SUBSTANCE ABUSE

A1	During the last 4 weeks how often have you had drinks containing alcohol? Would you say daily, weekly, less than weekly, or never? CIRCLE ONE CODE.	DAILY 1 WEEKLY 2 LESS THAN WEEKLY 3 NEVER 4
A1b	Can you recall an occasion or occasions, where you had six or more drinks?	NEVER 1 LESS THAN MONTHLY 2 MONTHLY 3 WEEKLY 4 DAILY OR ALMOST DAILY 5 REFUSE TO ANSWER:9
A2	In the past 6 months since (MONTH YEAR), have you.....	YES NO
		A. INJECTED ANY NON-PRESCRIPTION DRUGS? 1 2
		B. USED CRACK OR COCAINE? 1 2

	ASK EACH. CIRCLE YES OR NO FOR EACH.	C. USED GANJA?	1	2	
		D. USED ECSTACY &/OR MOLLY?	1	2	
		E. USED OTHER DRUG? State: _____			
A FEW QUESTIONS ABOUT VULNERABILITY					
A3	Have you ever: Circle all that apply		YES	NO	
		A. SPENT A NIGHT IN A JAIL OR PRISON?	1	2	
		B. BEEN RAPED?	1	2	
		C. SLEPT OUTDOORS OR IN A SHELTER BECAUSE YOU WERE HOMELESS?	1	2	
		D. BEEN VIOLENTLY ATTACKED BY A CLIENT	1	2	
		E. BEEN VIOLENTLY ATTACKED BY A SEX PARTNER	1	2	
		F. BEEN VIOLENTLY ATTACKED BY ANOTHER SEX WORKER	1	2	
EXPOSURE TO HIV/STI PREVENTION INTERVENTIONS					
A4	I want to know if you have been a part of any HIV/AIDS prevention Programmes in the past 6 months. In the past 6 months have you.....		YES	NO	
		A. PARTICIPATED IN A CONDOM DEMONSTRATION?	1	2	
		B. SEEN ANY AIDS PREVENTION MATERIALS (e.g. Posters, brochures, cards) AT YOUR CURRENT WORKSITE?	1	2	
		HAVE YOU TALKED WITH AN OUTREACH WORKER AT YOUR CURRENT WORK SITE ABOUT...?			
		C. YOUR RISK OF GETTING HIV/AIDS	YES	NO	NA
			1	2	9
	D. GETTING TESTED FOR HIV?	1	2	9	
	For Each Item, tell me if it is easy or hard for you to access the item when you need it:	Easy	Hard	DK	
	E Condoms	1	2	9	
	F Lube	1	2	9	
	G HIV Test	1	2	9	

	DO YOU THINK...?	YES	NO	DK
A5	A. Using condoms reduce the risk of HIV transmission?	1	2	9
	B. A healthy looking person can be infected with HIV?	1	2	9
	C. A person can get HIV from mosquito bites?	1	2	9
	D. A person can get HIV by sharing a meal with someone who is infected?	1	2	9
	E. Having sex with only one faithful, uninfected partner can reduce the risk of HIV transmission?	1	2	9
KNOWLEDGE OF PrEP				
A6	Have you ever heard of PrEP? (This is daily use of medications taking of pills by persons who are HIV negative and are at risk for HIV to reduce their risk of HIV infection) Skip to A10 if NO	Yes 1	No 2	
A7	Have you ever taken PrEP			
*HIV SELF-TESTING (HIV self-testing kits are testing kits that allow you to test yourself for HIV from the privacy of your home)				
A10	Have you ever heard about the HIV self- testing kit? Skip to H if NO	Yes 1	No 2	
A11	Have you ever used an HIV self-testing kit?	Yes 1	No 2	
NOW I WOULD LIKE TO ASK YOU A FEW MORE QUESTIONS TO FINISH THIS PART OF THE SURVEY.				
A6	How much risk do you think you have of getting HIV?	Great Risk 1 Moderate Risk 2 Little Risk 3 No Risk at All 4 Does not Know 5		
GO TO THE PRIVATE CARD				

QUESTIONNAIRE #: _____ PLACE SITE ID #: _____ PARTICIPANT CODE: _____

READ: I would like you to answer a few questions privately on a card that I will give you. I will read the question and you should write down your honest answer using a pen that I will give you. I'll read the questions and the answers. You should choose an answer and circle your response on the card. When we are finished with the card, you fold it and put it in this envelope. I cannot see your answers. Nobody can link the responses to you

so there is no reason to give a false answer. We really want to know the truth. Some of the questions may seem repetitive but please be patient.				
P 1	ASK EVERYONE: Are you willing and able to mark your answers on the card? END INTERVIEW IF ANSWER IS NO			YES 1 NO 2
GENDER BASED VIOLENCE				
	How frequently has the following occurred between you and your male partner/s	Never	Occasionally	Often
G1	Your partner insulted, harassed, embarrassed or tried to control you	1	2	3
G3	Your partner/s slapped hit or choked you.	1	2	3
G3	Your partner/s threatened you with any kind of weapon	1	2	3
G4	Your partner/s raped or forced you to have sex when you did not want to	1	2	3
P1	The last time you had sex with your main partner, did you and your partner use a condom? IF YOU DO NOT HAVE A MAIN PARTNER, CIRCLE NOT APPLICABLE.		YES 1 NO 2 NOT APPLICABLE 9	
P2	In the past 3 months, have you <u>been paid</u> money for sex? IF YOU HAVE NOT HAD SEX IN THE LAST 3 MONTHS CIRCLE NOT APPLICABLE.		YES 1 NO 2 NOT APPLICABLE 9	
P3	The last time you were paid money for sex, did you use a condom?		YES 1 NO 2 NOT APPLICABLE 9	
P4	What types of sex have you had in the past year? Have you had anal sex (Sex in the bottom)? If no to anal sex skip to P6		ORAL 1 ANAL 2 PeNIS TO VAGINA 3	
P5	The last time you had anal sex (Sex in the bottom), did you or the man you had sex with use a condom?		YES 1 NO 2	
P6	What was the result of your most recent HIV test not including the test today?		POSITIVE 1 NEGATIVE 2 never TESTed 3 don't Know/Did not get results 4	

P7	If your result was positive, have you ever registered for care and treatment at any health provider since your positive result? IF RESULT WAS NEGATIVE OR YOU NEVER TESTED FOR HIV CIRCLE 9 FOR NOT APPLICABLE	YES-PRIVATE DOCTOR 1 YES-PUBLIC HEALTH CENTRE 2 YES-BOTH PUBLIC AND PRIVATE DOCTORS 3 NEVER REGISTERED 4 NOT APPLICABLE 9	
P8	If you received a HIV positive result and have registered for care and treatment of HIV , have you visited a clinic/doctor for treatment in the last 12 months	YES 1 NO 2 NOT APPLICABLE 9	
P9	If No, what are the reasons for you not going to a doctor or clinic in the last 12 months?	FEEL HEALTHY 1 FEAR BEING TREATED BADLY BY CLINIC STAFF/HEALTH CARE WORKERS 2 DON'T WANT OTHERS TO KNOW OF MY INFECTION 3 TOO LONG WAITING TIME AT CLINIC 4 NOT APPLICABLE 9	
P10	How long after receiving your positive HIV result did you go to a clinic/doctor?	WITHIN ONE MONTH 1 WITHIN 3 MONTHS 2 WITHIN ONE YEAR 3 LONGER THAN ONE YEAR 4 NOT APPLICABLE 9	
P11	What is the main reason for you not registering at a clinic or doctor after receiving your positive HIV result?	FEEL HEALTHY 1 FEAR BEING TREATED BADLY BY CLINIC STAFF/HEALTH CARE WORKERS 2 DON'T WANT OTHERS TO KNOW OF MY INFECTION 3 TOO LONG WAITING TIME AT CLINIC 4 NOT APPLICABLE 9	
P12	Have you ever started anti-retroviral therapy (ART)? (This is medication prescribed by a health care provider to reduce the amount of HIV in your body and help you stay healthy)	Yes 1	No 2
P14	Are you currently on ART?	Yes 1	No 2
P14A	If No, what are the reasons for you not accepting ART?	FEEL HEALTHY 1	

	Circle all that apply END INTERVIEW IF NOT ON ART	FEAR BEING TREATED BADLY BY CLINIC STAFF/HEALTH CARE WORKERS 2 DON'T WANT OTHERS TO KNOW OF MY INFECTION 3 TOO LONG WAITING TIME AT CLINIC 4 FEAR THE SIDE EFFECTS OF THE MEDICATIONS 5 NOT APPLICABLE 9	
P1 5	When did you last visit/ check up at your clinic or doctor to follow-up monitor your ART?	IN THE LAST 3 MONTHS 1 IN THE LAST 6 MONTHS 2 IN THE LAST ONE YEAR 3 LONGER THAN ONE YEAR 4 NOT APPLICABLE 9	
P1 6	When did you do your viral load blood test?	IN THE LAST YEAR 1 LONGER THAN ONE YEAR 2 NOT APPLICABLE 9	
P1 7	Was your viral load undetectable at your last test?	Yes 1	No 2
THANK YOU VERY MUCH FOR PARTICIPATING. NOW YOU WILL BE TESTED FOR HIV AND SYPHILIS			
TIME ENDED: HOUR:___/___ MINUTE___/___ AM / PM Checked by Supervisor: _____ Date: _____ Entered by Data entry person: _____			
INTERVIEWER COMMENTS.			
CONFIRM Q8	PERSON CONFIRMED AS:	1. STREET SEX WORKER 2. OTHER SEX WORKER (CLUB, MASSAGE PALOURS, ONLINE, TELEPHONE ETC.) 3. WORKER (BARTENDER, WAITRESS, ETC.) 4. PATRON	
	HIV Result		
	Syphilis Result		

